



Lord and Lady of the Cemetery (late 19th–early 20th century) in Nechung Chapel.
Photograph: Thomas Laird

Has examples of

Local adaptation

Divergence

**Homeostatic &
acclimatization
responses**

Maladaptation

*Contrasting results
in ambiguous cases
of specific adaptation*

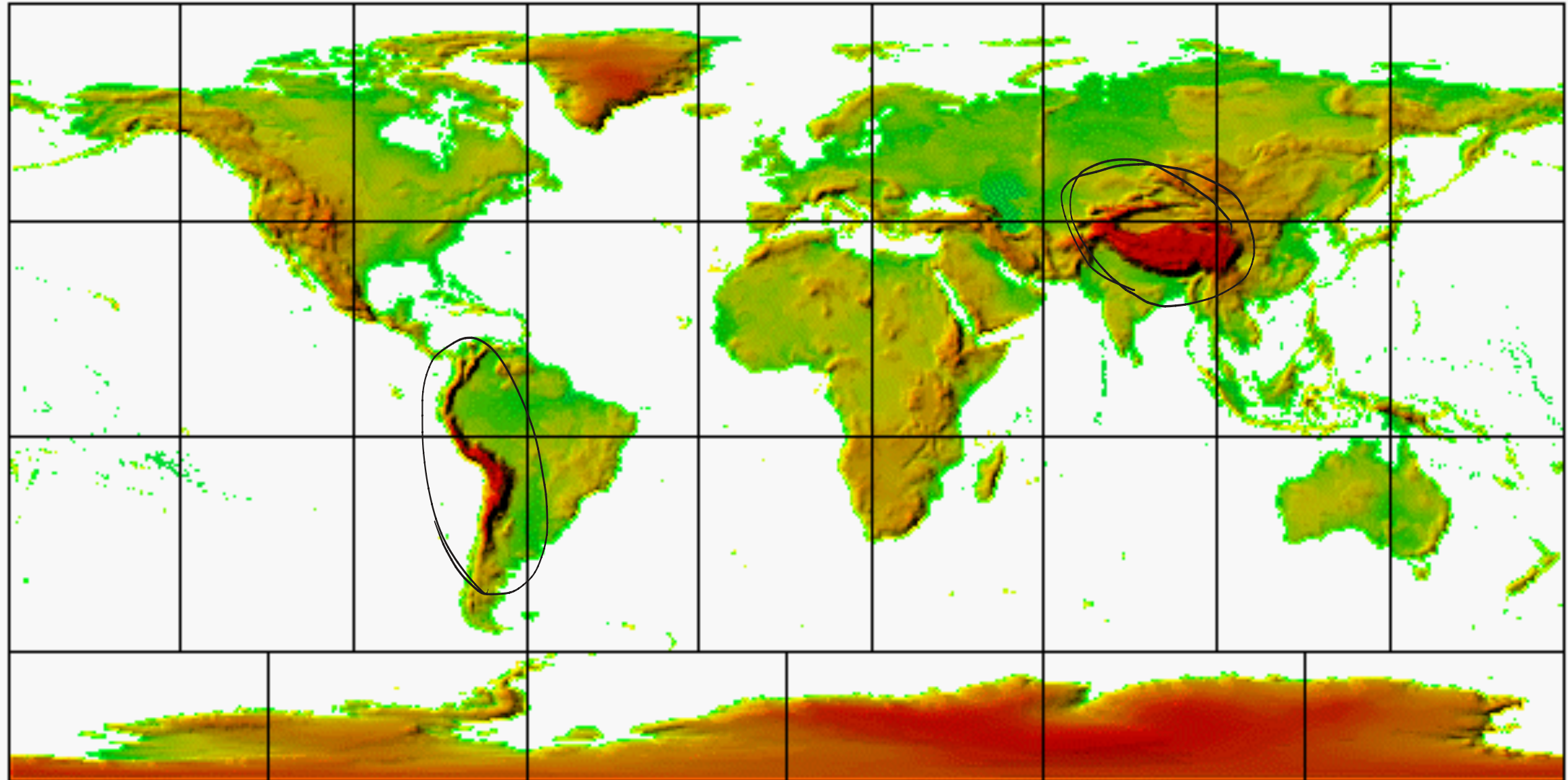
**Human
evolutionary
history**

High altitude adaptation

Dr. Laura van Holstein

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Room 215

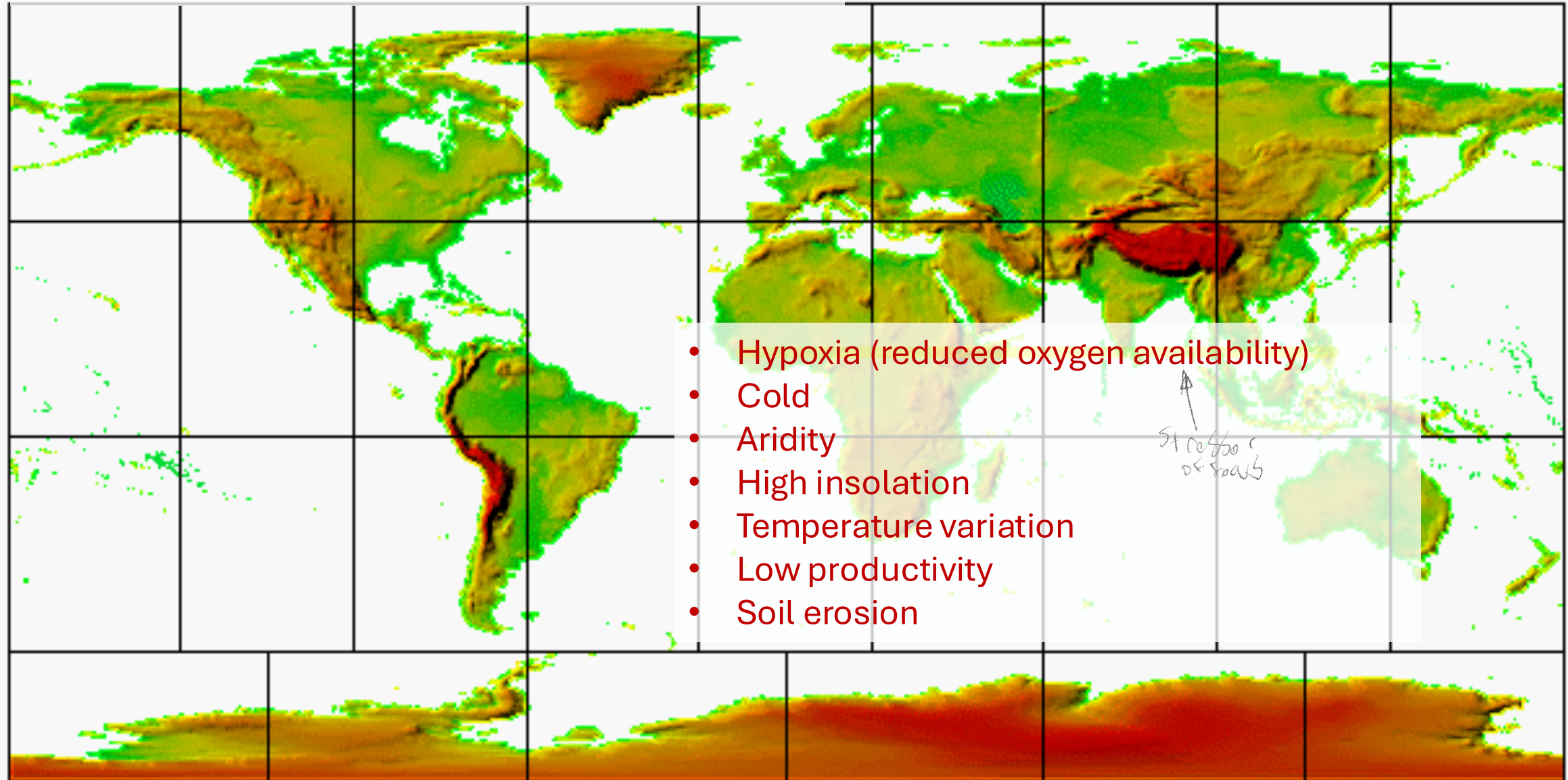


The problem

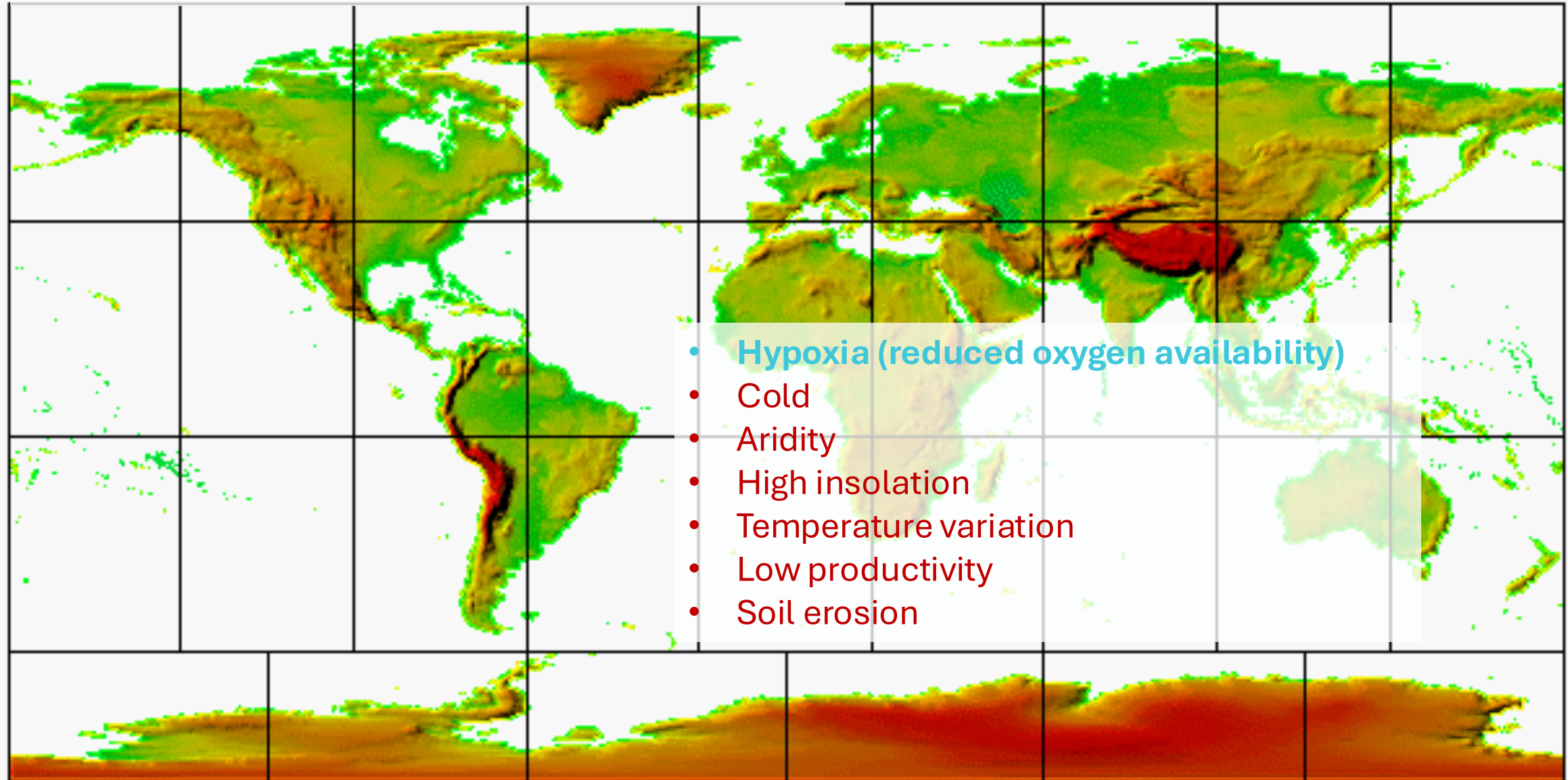
Your solutions

Natural selection's solutions

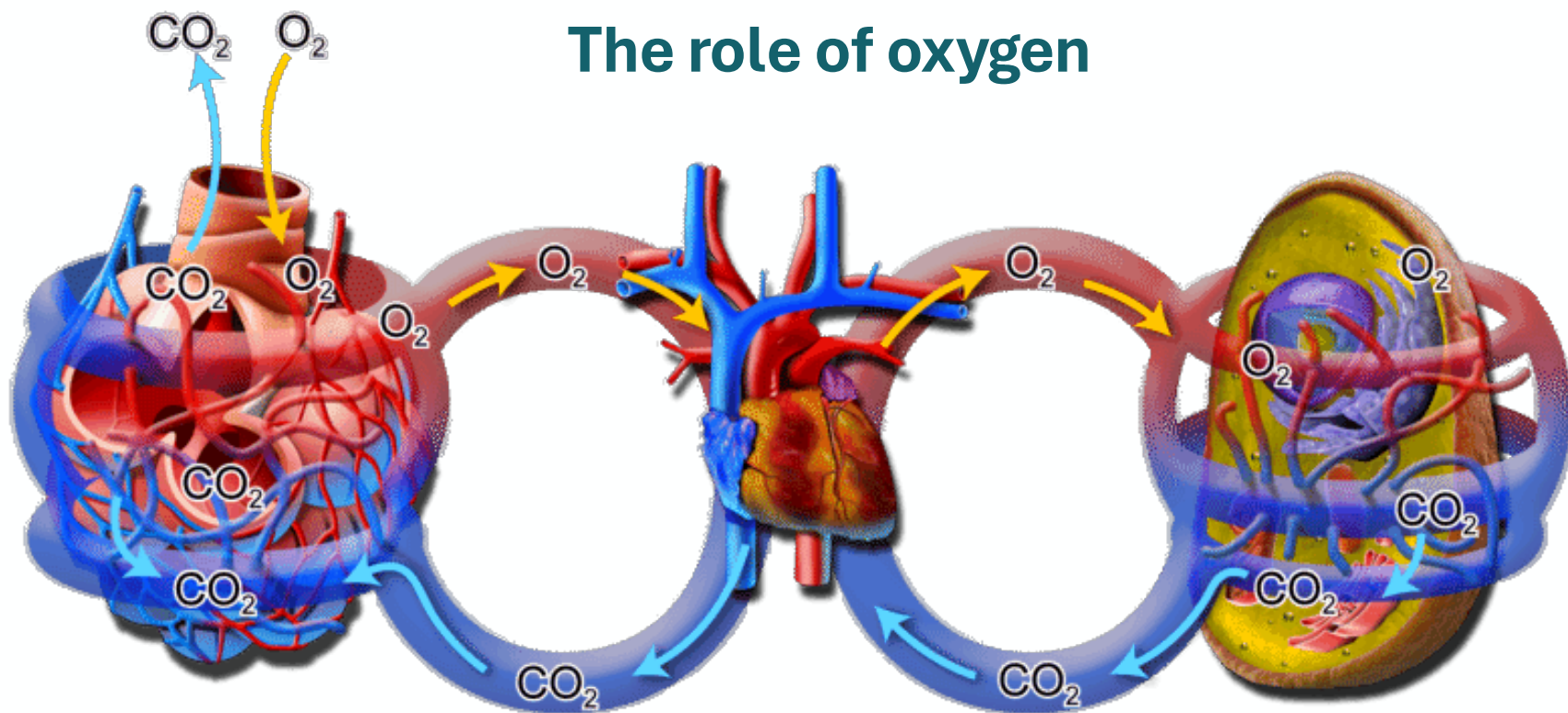
High altitude: many stressors



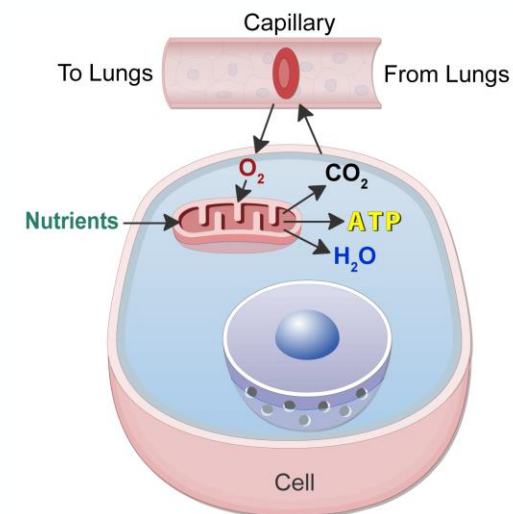
High altitude: many stressors



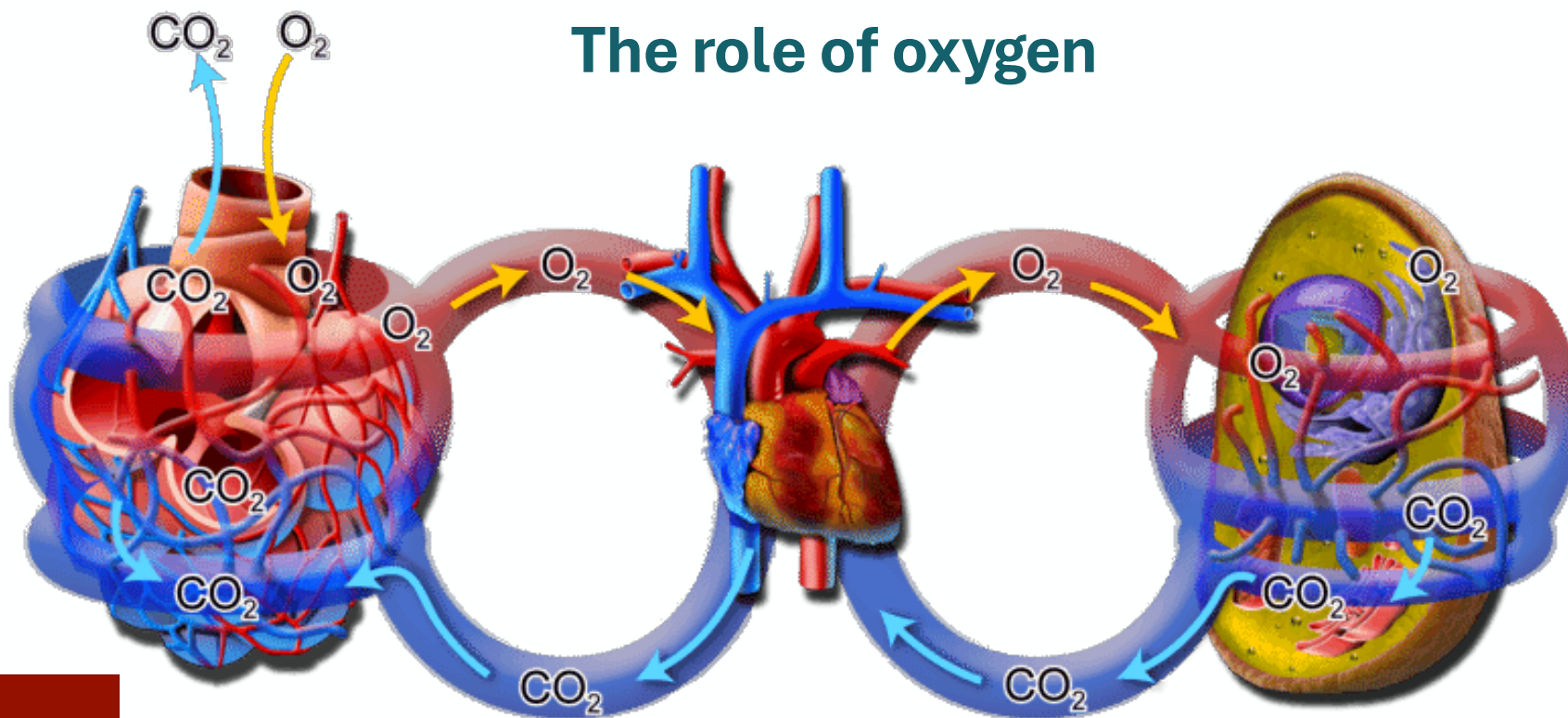
The role of oxygen



Alveolus
surrounded by
capillaries



The role of oxygen

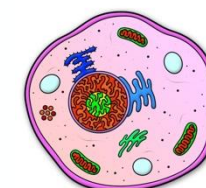


Ambient oxygen pressure
(Ambient PO_2)

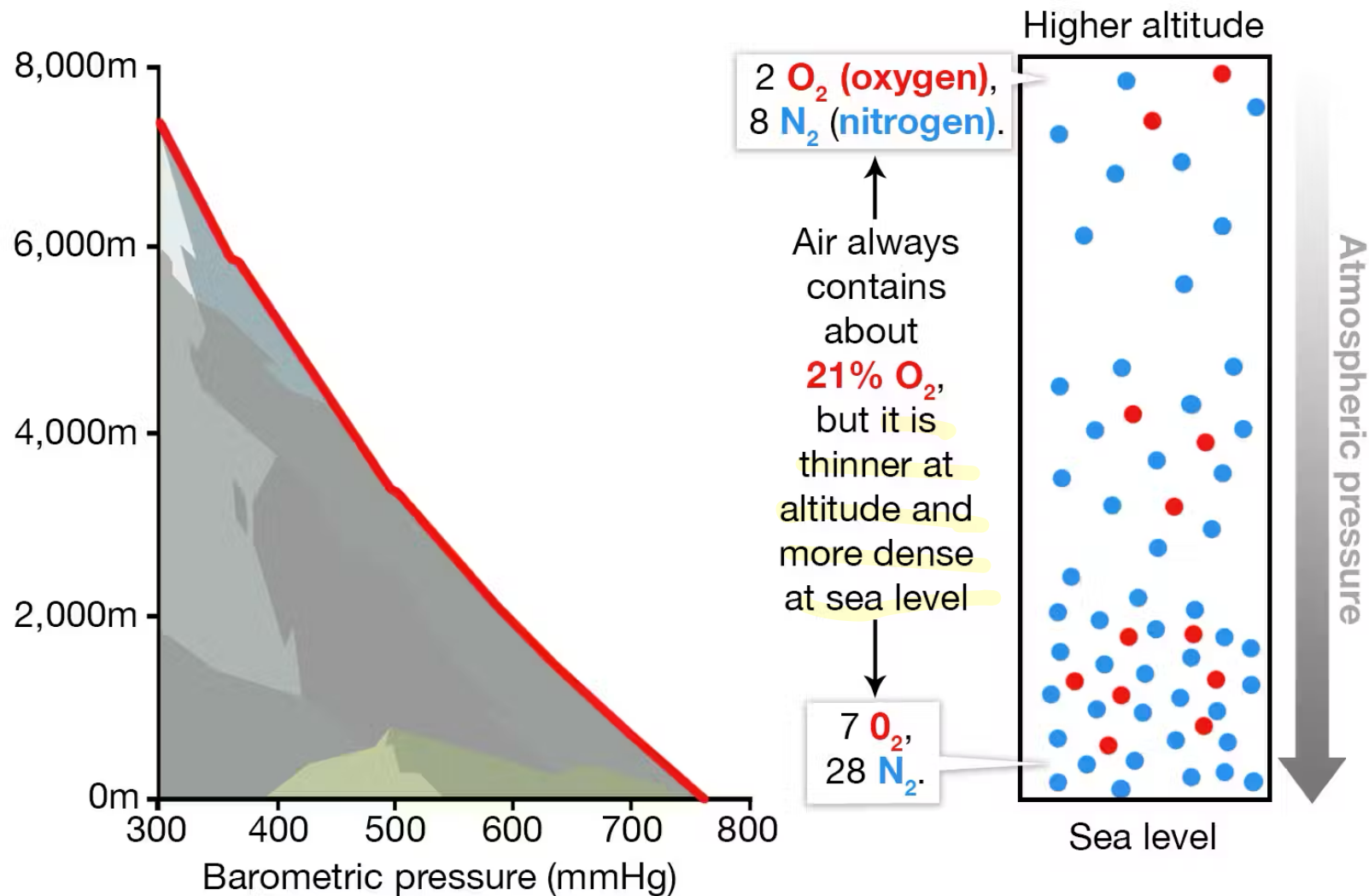
Alveolar oxygen pressure (P_AO_2)

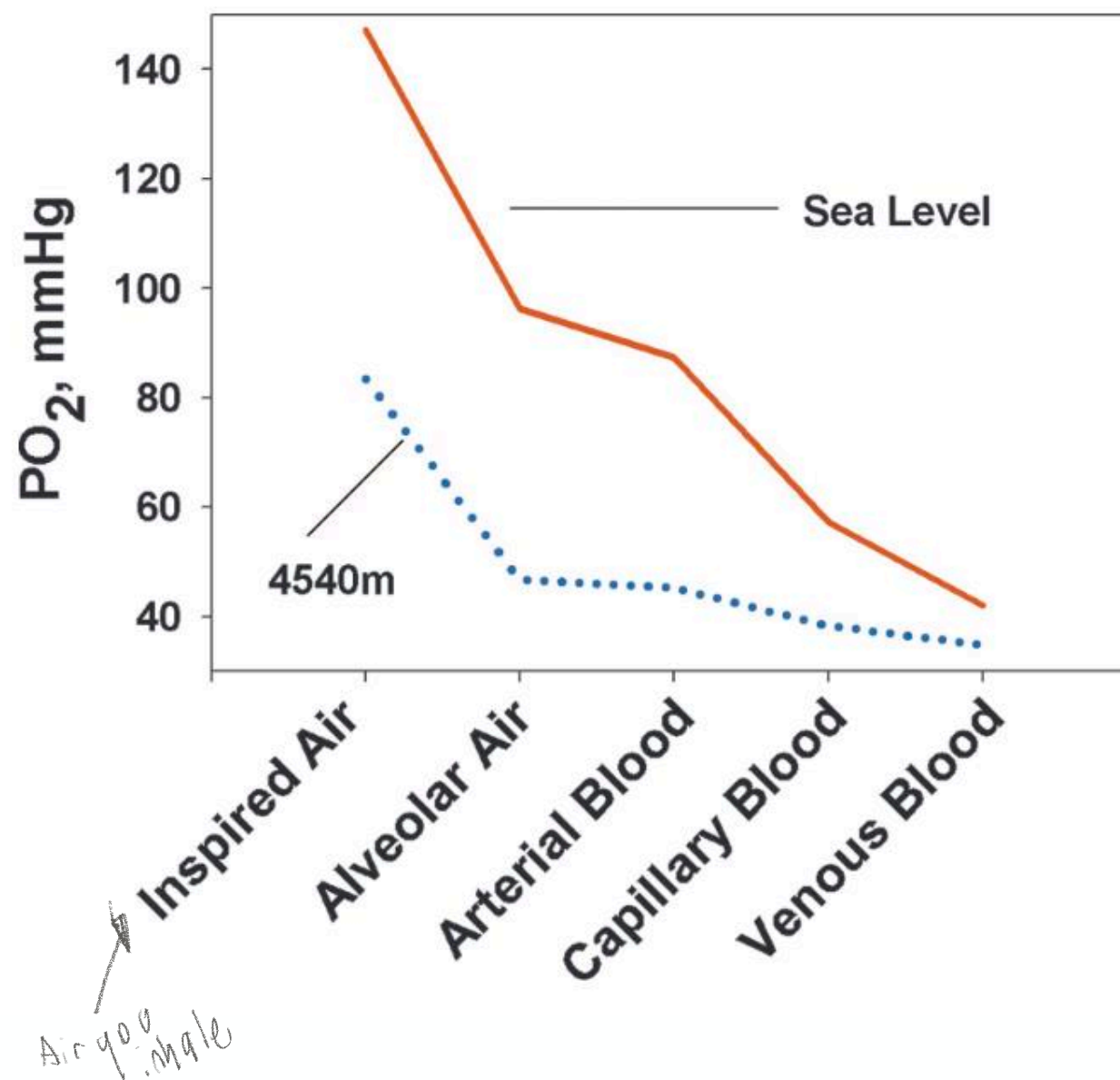
Saturation of haemoglobin (SaO_2)

O_2 content of blood per liter = $SaO_2 + Hb + \text{blood flow}$



The impact of altitude on oxygen levels



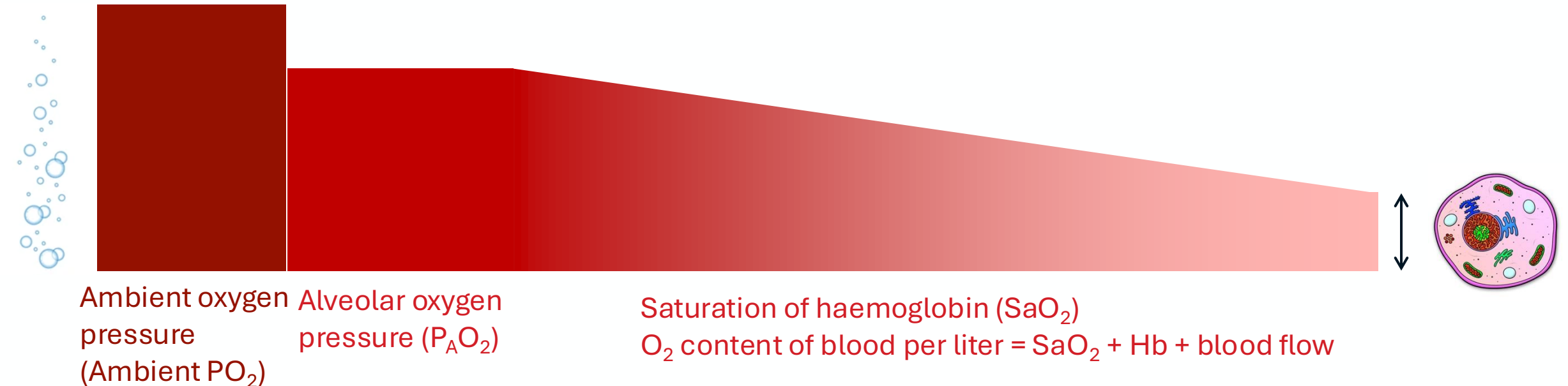


The role of oxygen

The problem

Your solutions

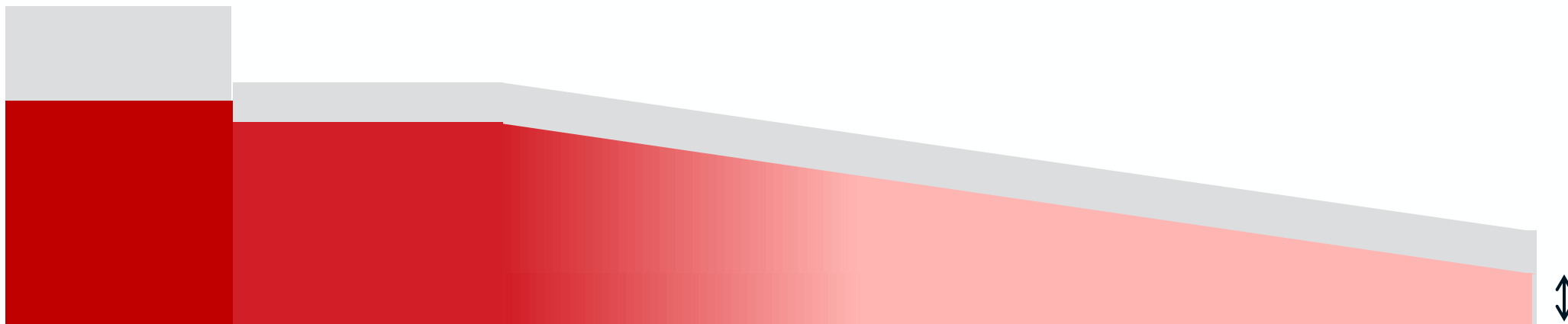
Natural selection's solutions



The role of oxygen

Less oxygen available
and diffusion slows down
due to smaller
pressure differences

Under hypoxia: the pressure differences among different stages of oxygen transport are smaller: diffusion rate is lower.



Ambient oxygen pressure
(Ambient PO_2)

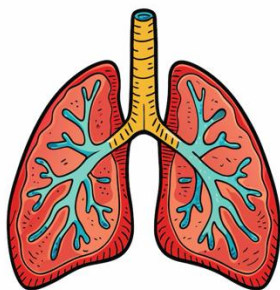
Alveolar oxygen pressure (P_AO_2)

Saturation of haemoglobin (SaO_2)

O_2 content of blood per liter = $SaO_2 + Hb + \text{blood flow}$

What now?!

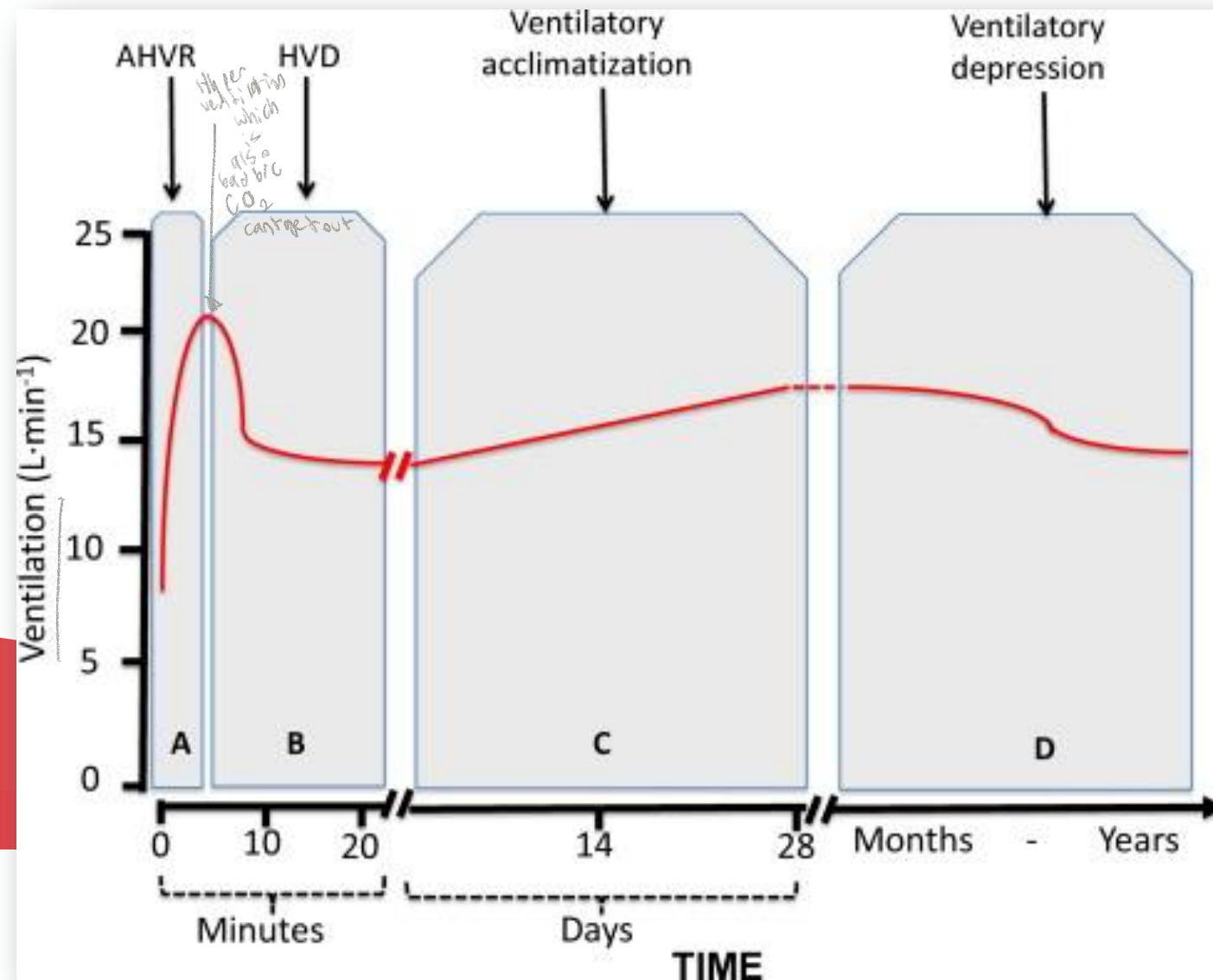
Increase breathing rate



Breathing rate

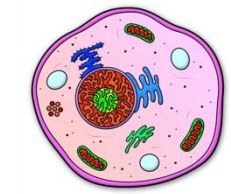
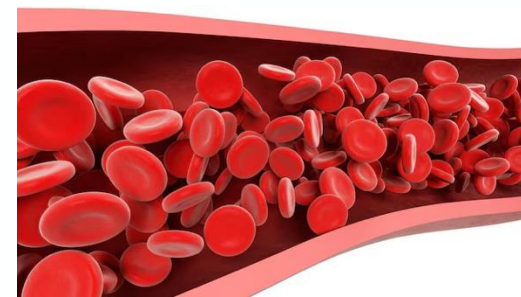
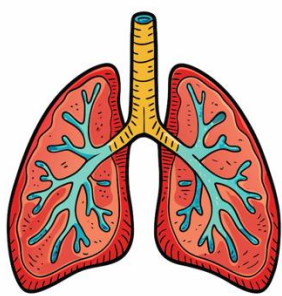
Ambient oxygen pressure
(Ambient PO_2)

Alveolar oxygen pressure (P_AO_2)



What now?!

Increase breathing rate



Ambient oxygen pressure (Ambient PO_2)

Alveolar oxygen pressure ($P_A O_2$)

Saturation of haemoglobin (SaO_2)

O_2 content of blood per liter = $SaO_2 + Hb + \text{blood flow}$

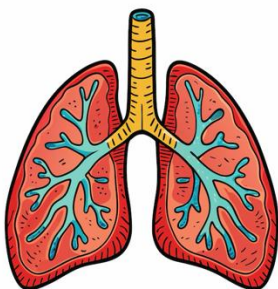
diff. parts to act on to increase differential

of RBCs

↑ such as Hb increase

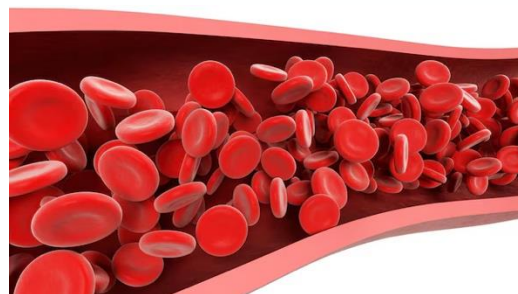
What now?!

Increase breathing rate



Increase haemoglobin

Increase blood flow

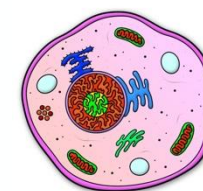


Ambient oxygen pressure
(Ambient PO_2)

Alveolar oxygen pressure (P_AO_2)

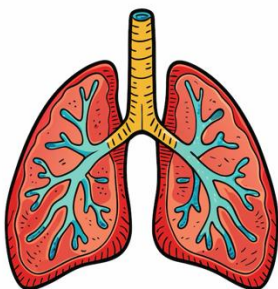
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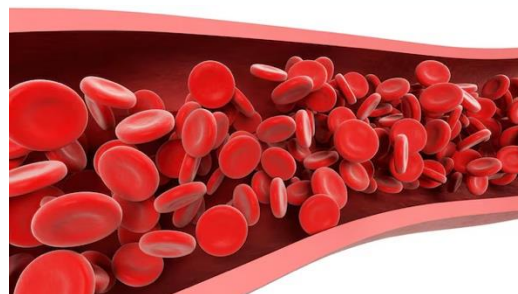


What now?!

Increase breathing rate



Increase haemoglobin
Increase blood flow

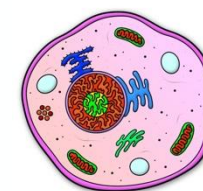


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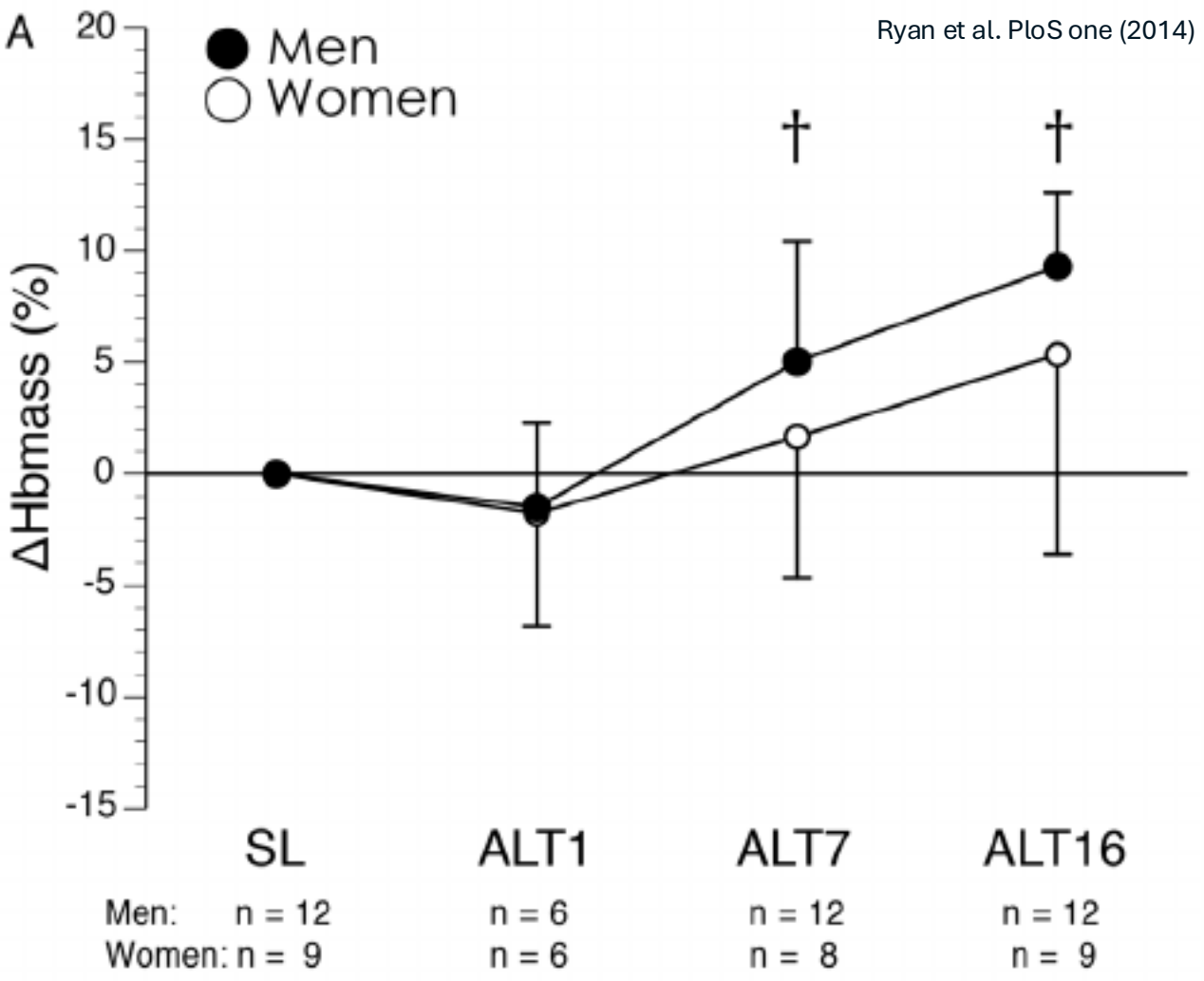
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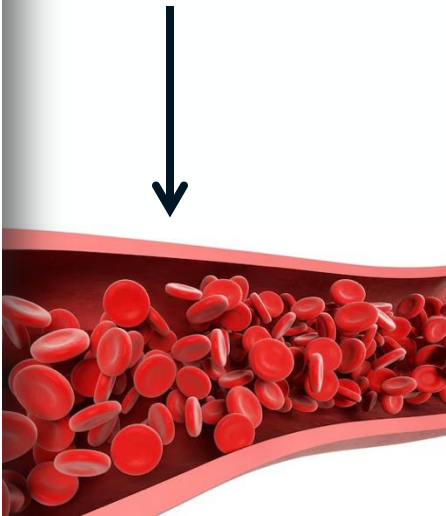


What now?!



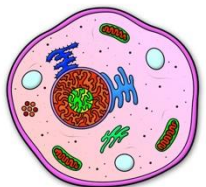
Increase haemoglobin

Increase blood flow

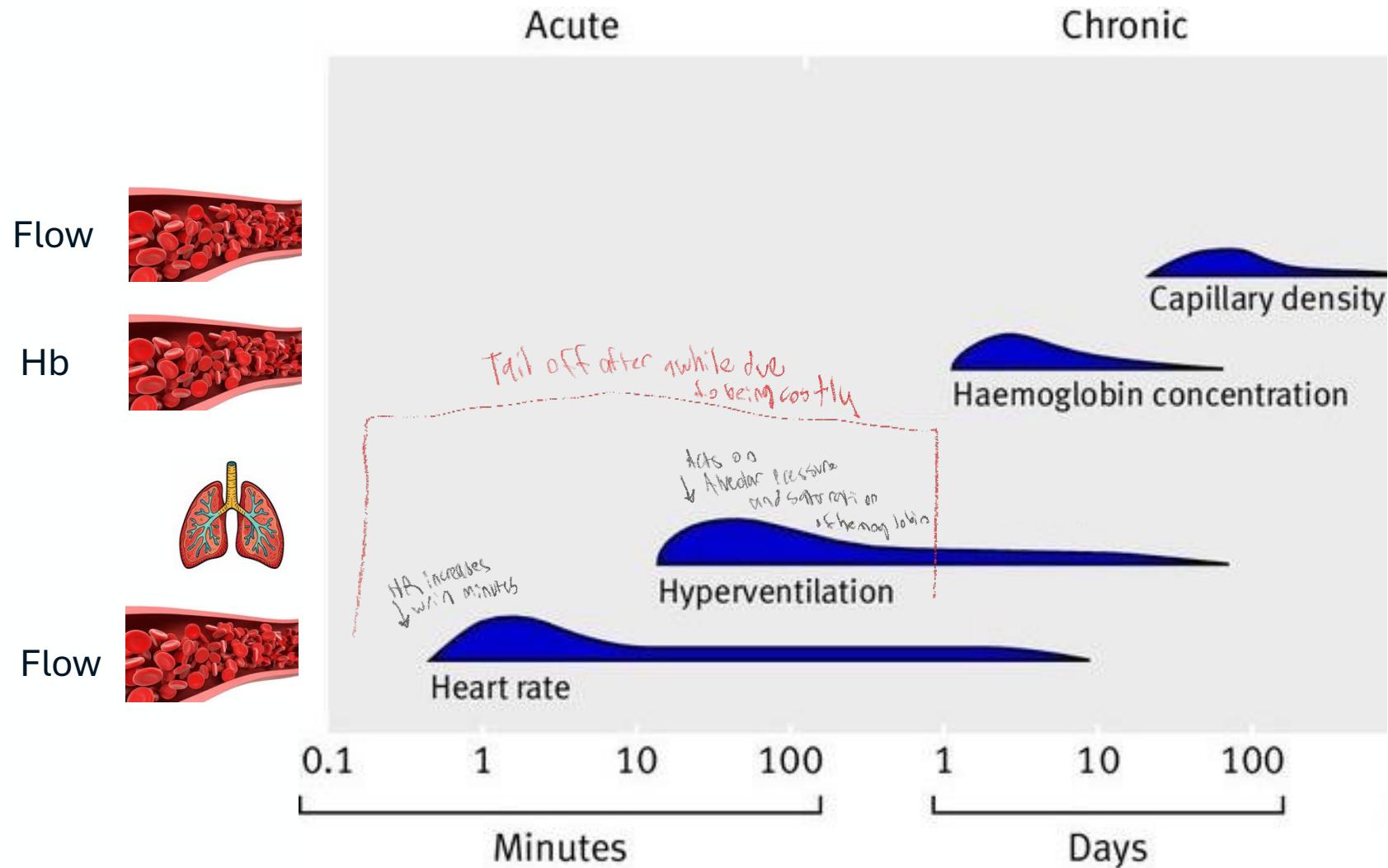


haemoglobin (SaO₂)

blood per liter = SaO₂ + Hb + blood flow



What now?!



Hypoxia-inducible factors

↑ Transcription factor
proteins that
act on
expression
of genes

Degradation, dependent on
oxygen availability

EGLN1

codes for PHD2

PHD2

PHD2
deg codes HIFs
depending on
oxygen availability

HIF1A

EPAS1

codes for

HIF2A

HIF3A

Transcription
factors

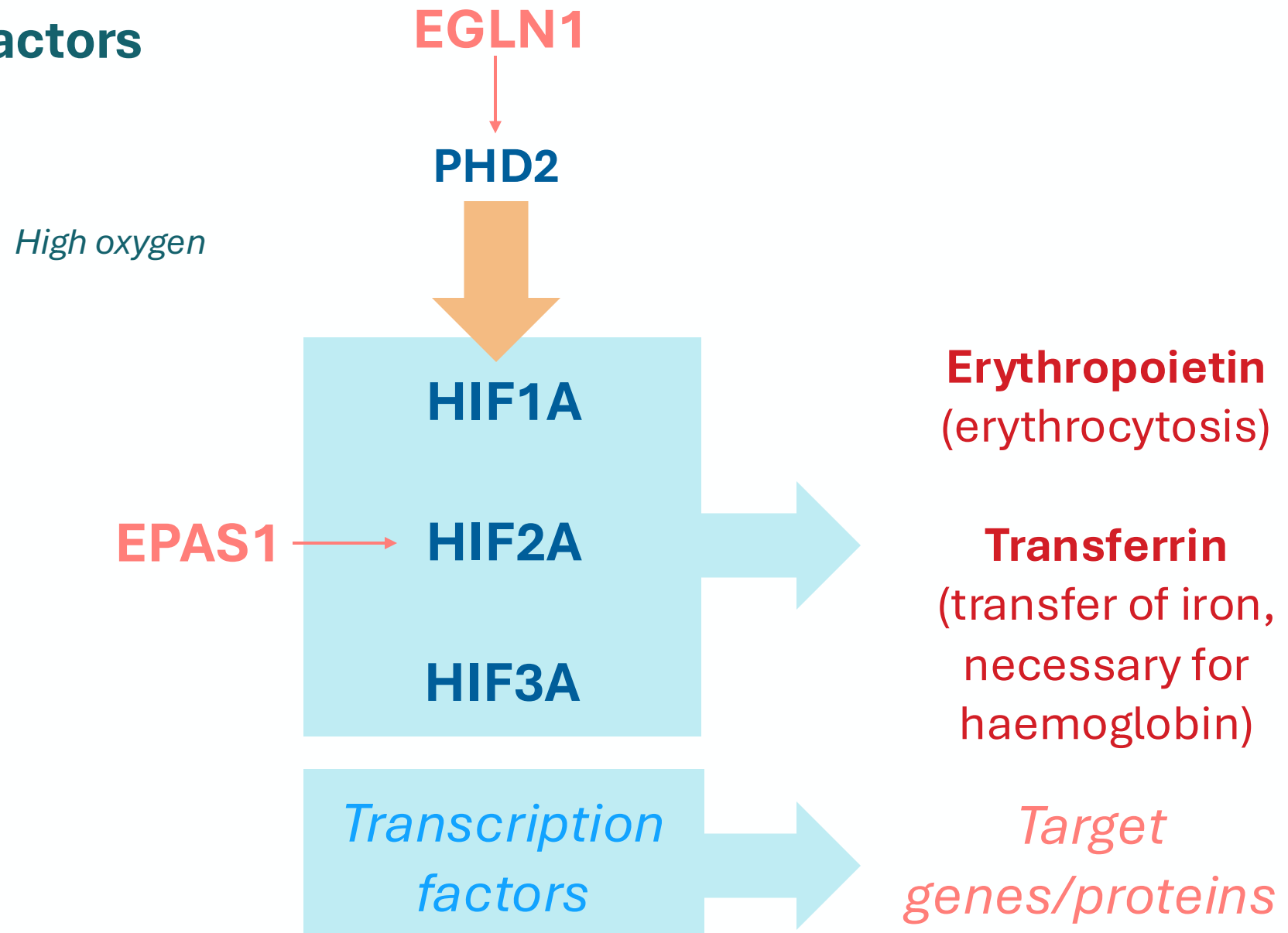
Protein helps form RBCs
Erythropoietin
(erythrocytosis)

Transferrin
(transfer of iron,
necessary for
haemoglobin)

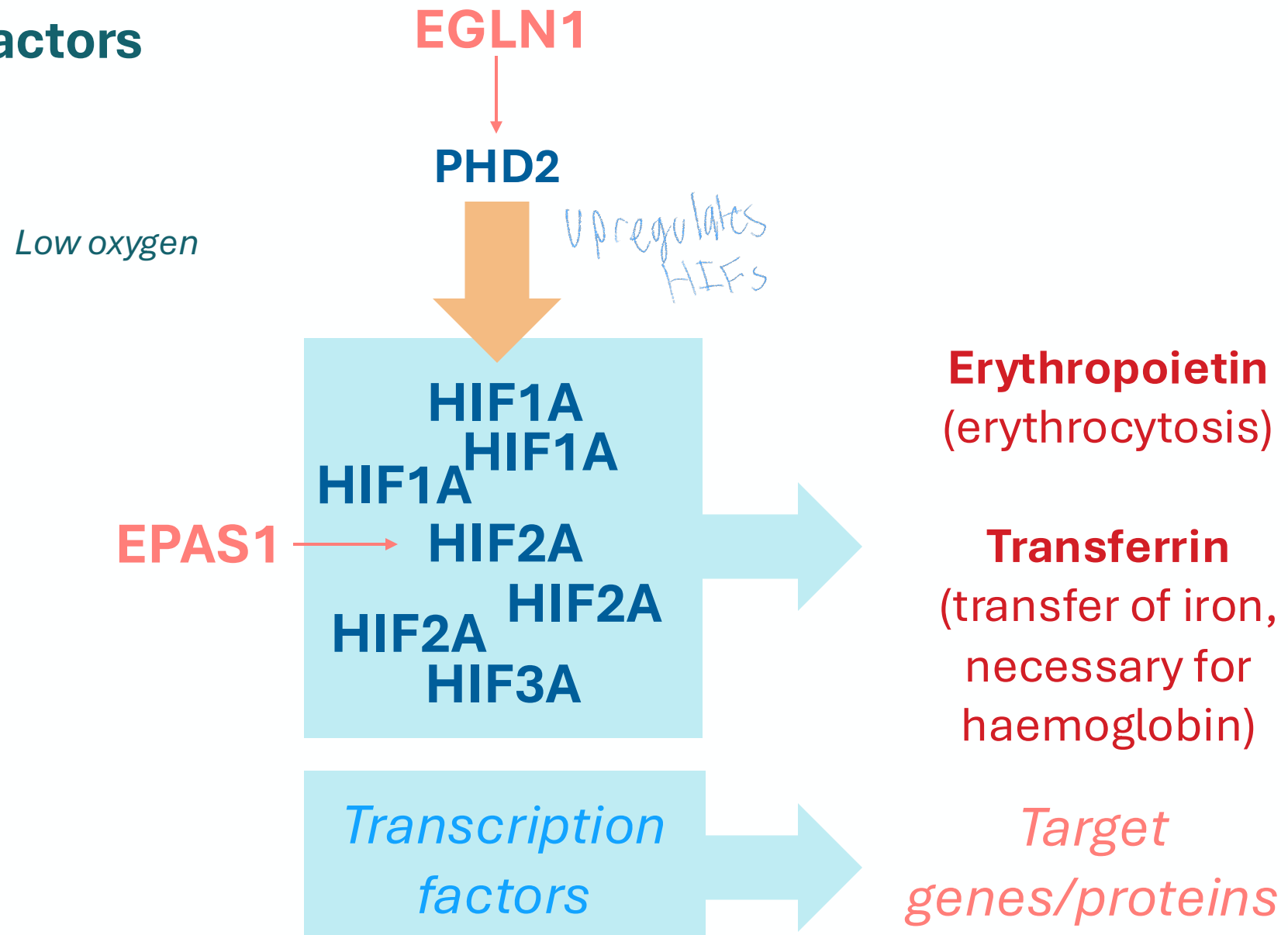
Target
genes/proteins

Regulates
production
of

Hypoxia-inducible factors



Hypoxia-inducible factors

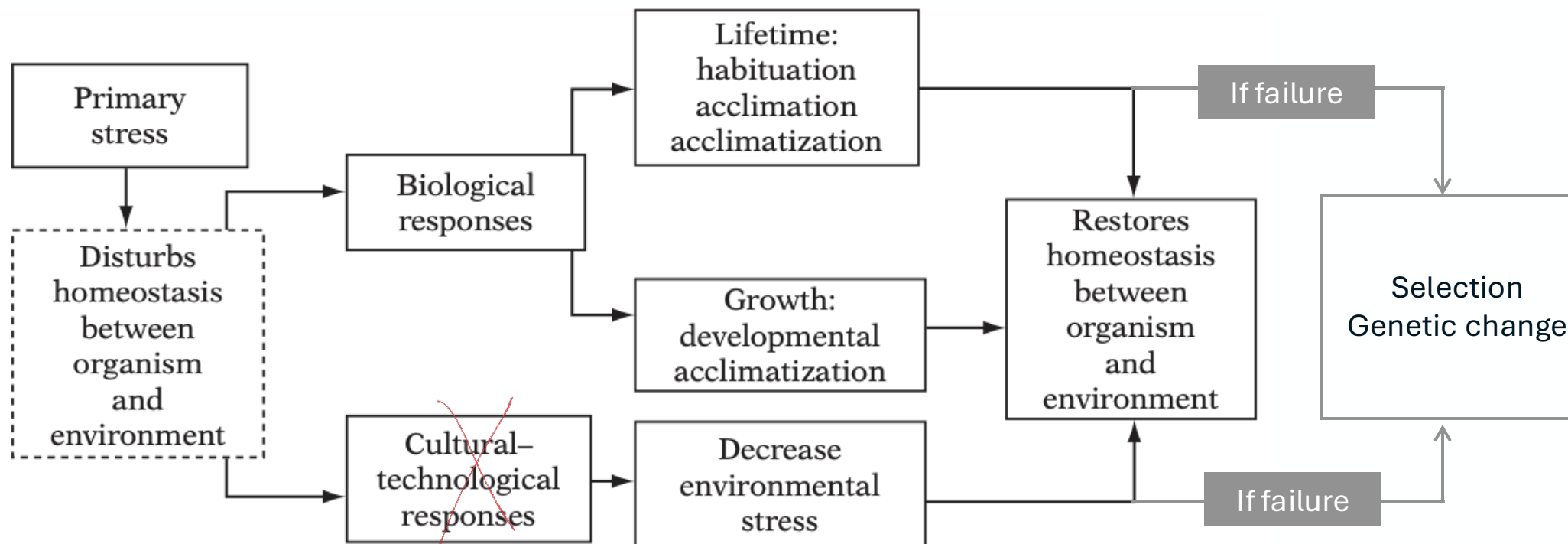


Recall lecture 3

The problem

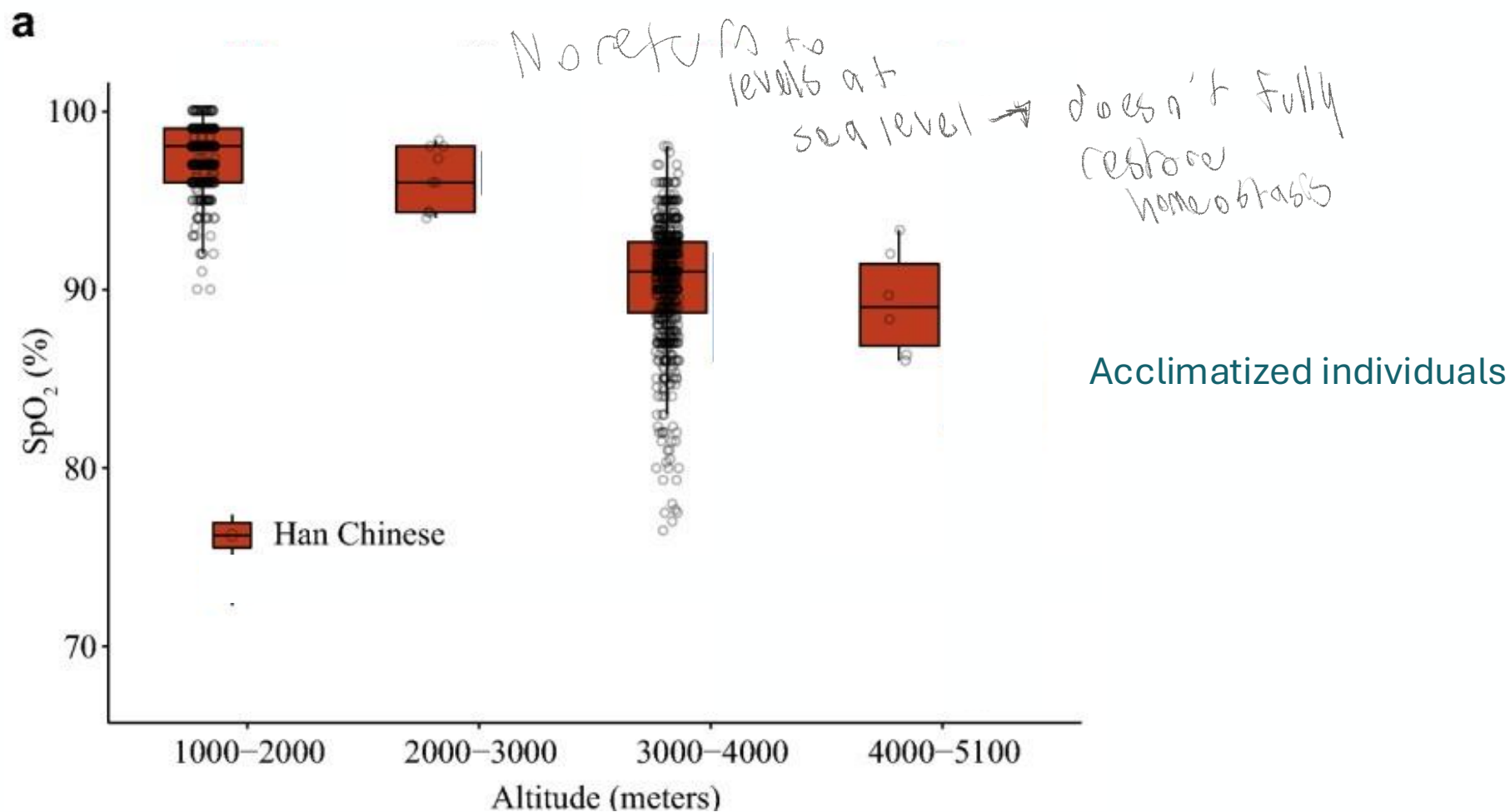
Your solutions

Natural selection's solutions

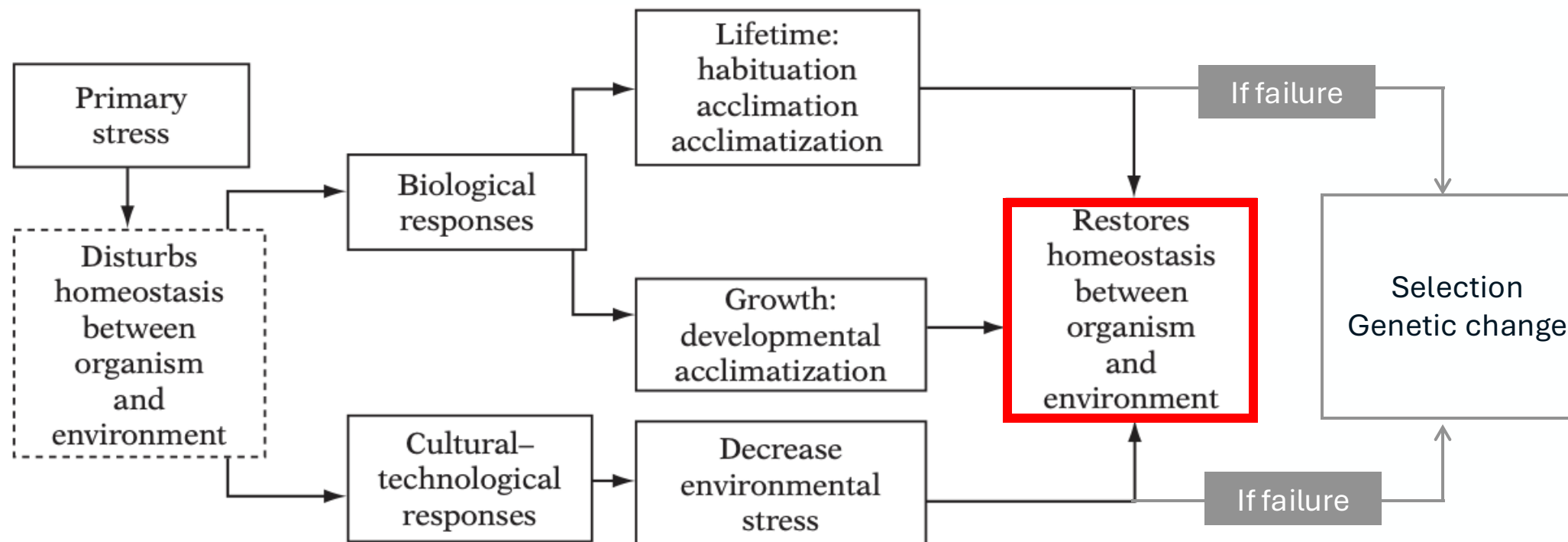


won't
help at high
altitudes
for oxygen tank

Effectiveness & costs of these responses



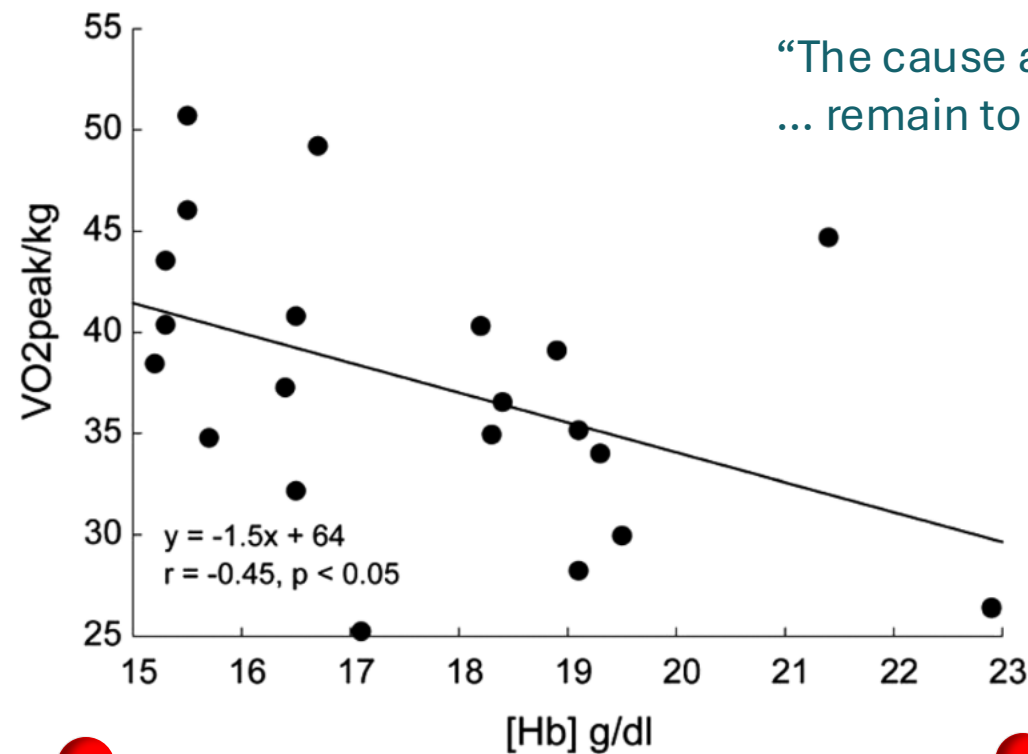
Recall lecture 3



Effectiveness & costs of these responses

- Reduced peak exercise capacity

Peak exercise capacity



Haemoglobin concentration

Effectiveness & costs of these responses

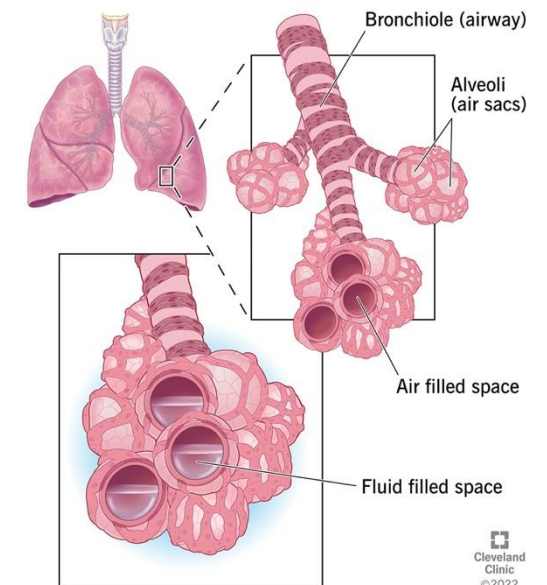
- Reduced peak exercise capacity
- Chronic Mountain Sickness; other cardiovascular diseases

Chronic Mountain Sickness



Excessive erythrocytosis:

- Hypertension
- Risk of pulmonary and cerebral oedema (fluid build-up in lungs and brain)
- Risk of stroke



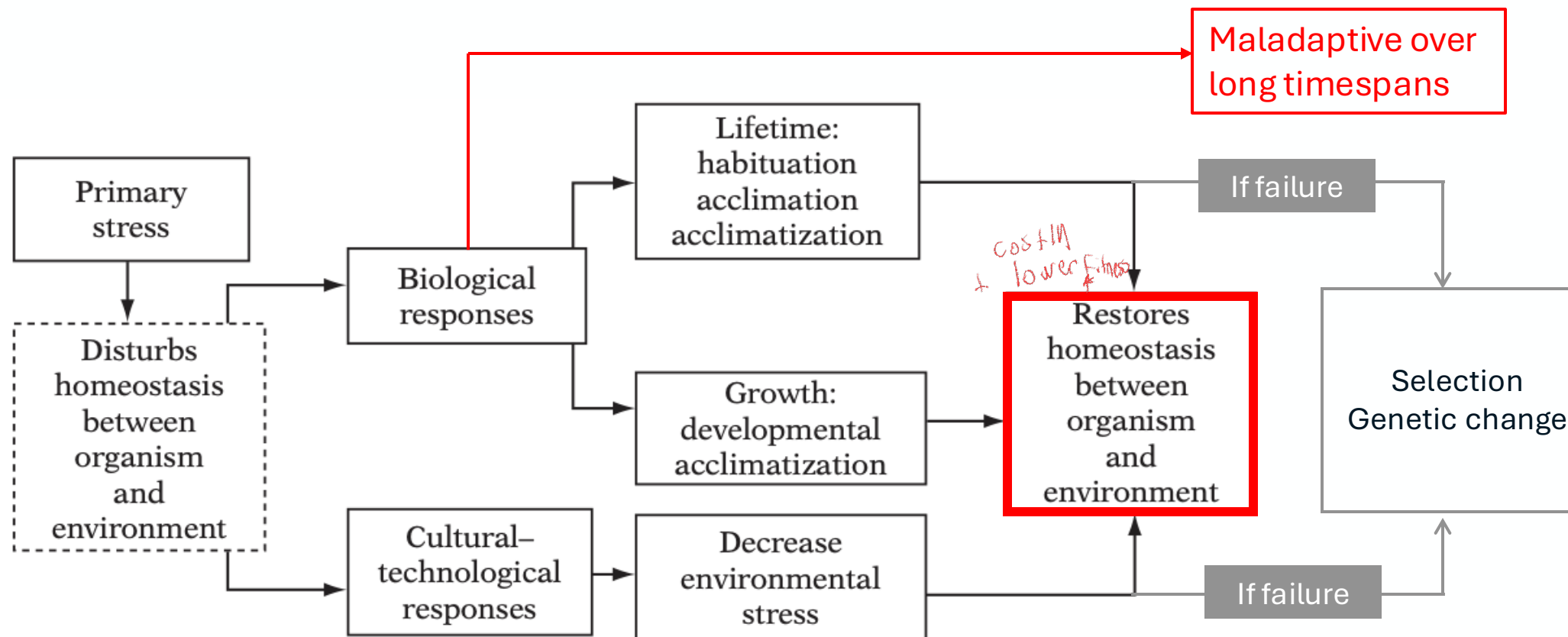
Effectiveness & costs of these responses

- Reduced peak exercise capacity
- Chronic Mountain Sickness; other cardiovascular diseases
- Poorer reproductive outcomes

Continuous variable	N	Mean	SD	Min	Max
Pregnancies, #	1006	5.6	2.86	0	15
Live births, #	1006	5.4	2.77	0	14
Children Surviving to the age of 15 years, #	979	4.0	2.14	0	11
Age, years	1006	55	11	39	91
Age at first pregnancy and birth, years	982	24	5	15	46
Age at last pregnancy and birth*, years	982	37	6	19	58
Hemoglobin concentration, gm/dl	959	13.8	1.51	5.2	18.7
O2 Saturation, %	959	87.4	4.5	60	96
Pulse, f/min	958	74	11	49	121
Categorical variable	Count (%)		# Missing (%)		
Current marital status	1 Married	659 (65.51)	10 (0.99)		
	2 Widowed	252 (25.05)			
	3 Divorced/separate	40 (3.98)			
	4 Never married	55 (5.47)			
Use of contraception	0 Never	682 (67.79)			
	1 Past use	147 (14.61)			
	2 Current	167 (16.6)			
District of residence	1 Gorkha	264 (26.24)			
	2 Mustang	742 (73.76)			
Type of marriage	0 Never	55 (5.47)			
	1 Cousin, not polyandrous	72 (7.16)			
	2 Not cousin, is polyandrous	94 (9.34)			
	3 Cousin and polyandrous	16 (1.59)			
	4 Not cousin, not polyandrous	769 (76.44)			

Lower haemoglobin content:
better reproductive outcomes

Recall lecture 3



Adaptive responses to high-altitude hypoxia

↑ Increase; costly

Homeostasis +
acclimation

	Sojourners
Resting ventilation	↑
Hb concentration	↑
Blood flow	↑
Capillary density	↑

people born
at low
altitudes
who go to
high altitudes

Prevent short-term risk to life...

But at **high long-term cost**

Birth weight Offspring survival	↓
------------------------------------	---

And not even that effective...

Adaptive responses to high-altitude hypoxia

↑ Increase; costly

Homeostasis +
acclimation

	Sojourners
Resting ventilation	↑
Hb concentration	↑
Blood flow	↑
Capillary density	↑

A 50-year period of infertility in early Spaniards in the Andes at 4100m!

Preexisting Medical Conditions

People with pre-existing medical conditions should talk with a doctor before traveling to high elevation.

- People with heart or lung disease should talk to a doctor who is familiar with high-altitude medicine before their trip.
- People with diabetes need to be aware that their illness may be difficult to manage at high elevation.
- Pregnant women can make brief trips to high elevations but they should talk with their doctor because they may be advised not to sleep at elevations above 10,000 feet.
- People with some illnesses (e.g., sickle cell anemia, severe pulmonary hypertension) should not travel to high elevations under any circumstances.



Birth weight Offspring survival	↓
------------------------------------	---

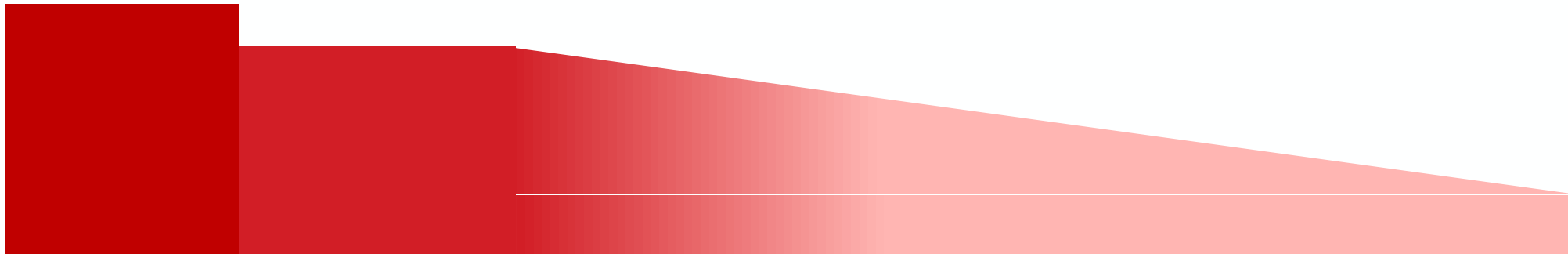
Fitness costs: natural selection

With falling barometric pressure comes rising evolutionary pressure

The problem

Your solutions

Natural selection's solutions

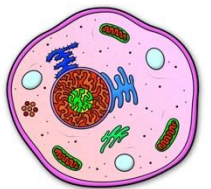


Ambient oxygen pressure
(Ambient PO_2)

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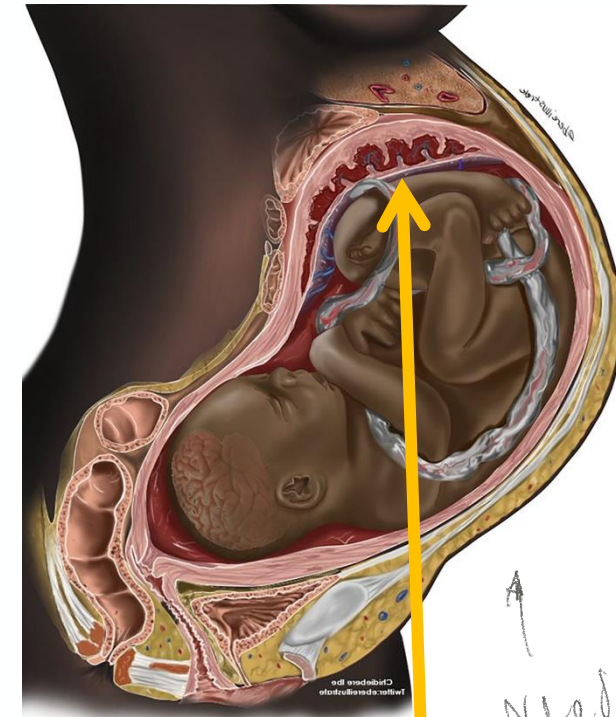


With falling barometric pressure comes rising evolutionary pressure

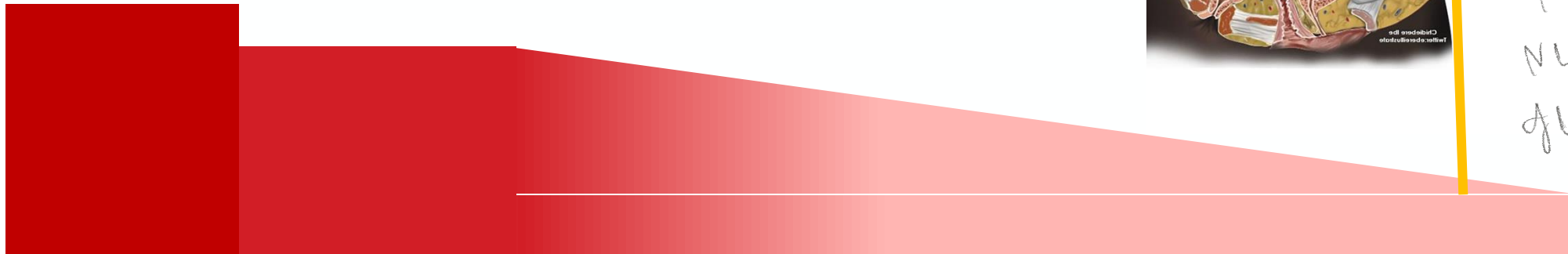
The problem

Your solutions

Natural selection's solutions



Need to
get oxygen
into
developing
fetus

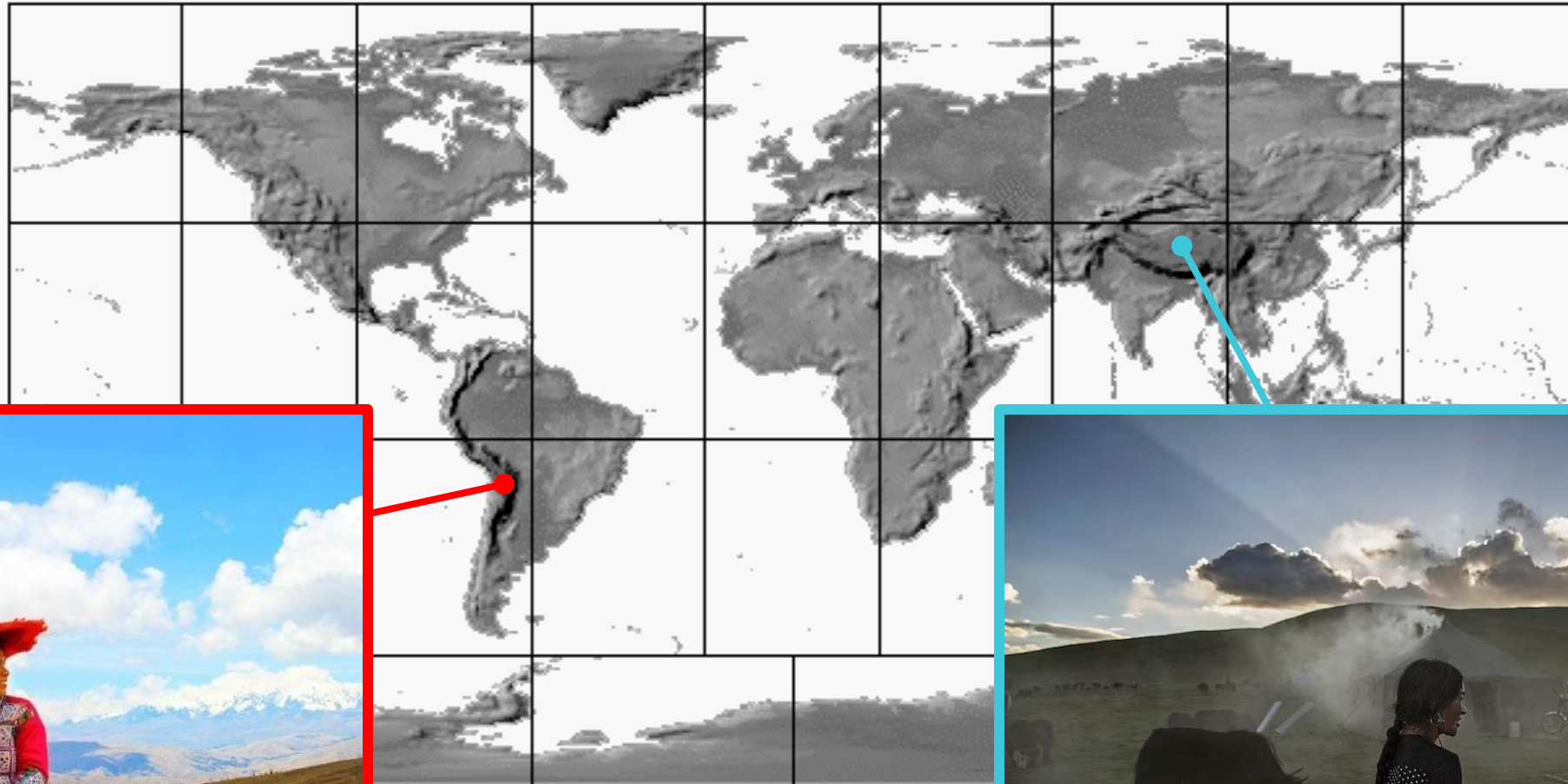


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Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

	Sojourners	Tibetans	Andeans
Resting ventilation	↑		
Hb concentration	↑		
Blood flow	↑		
Capillary density	↑		
Birth weight Offspring survival	↓	—	—

Adaptive responses to high-altitude hypoxia

Homeostasis + acclimation

Same stress in 2 different pops - Tibetans mix w/ Denisovans who are likely adapted + Tibetans have more time to adapt + local adaptation takes along time & recovery adaptive responses

High altitude one of the strongest selective pressures on genome along w/ disease

	Sojourners	Tibetans	Andeans
Resting ventilation	↑	You have been assigned to one of these two populations. Evolve, over the course of the appropriate generations, the ability to have higher birthweight than sojourners. You start with the same acclimatizing responses as sojourners – which are driven by the HIF pathway, and which are, clearly, maladaptive over the course of a lifetime. How do you counter these? Do you... • suppress the hypoxic response and evolve other ways to increase oxygen delivery to your tissues and those of the foetus? • permit the hypoxic responses and evolve adaptations to counter the evolutionary costs of these responses? • find some other solution?	
Hb concentration	↑		
Blood flow	↑		
Capillary density	↑		
Birth weight	↓	—	—
Offspring survival			

reaction b/w immediate survival but there are taking negative effects - otherwise release & come up w something else

Think abt tradeoffs for different

The problem

Your solutions

Natural selection's solutions

Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

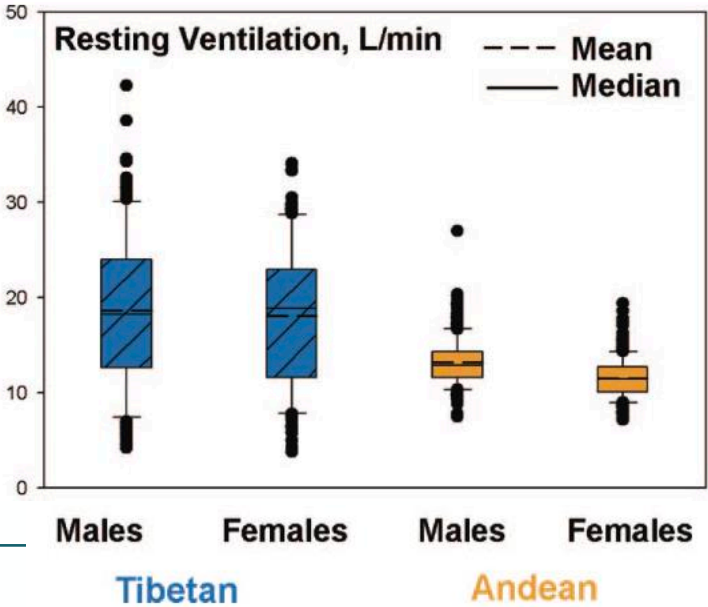
	Sojourners	Tibetans	Andeans
Resting ventilation	↑		
Hb concentration	↑		
Blood flow	↑		
Capillary density	↑		
Birth weight Offspring survival	↓	—	—

Adaptive responses to high-altitude hypoxia

Homeostasis + acclimation

Means same as baseline at sea level for people born at sea level while sojourners arrive at sea level

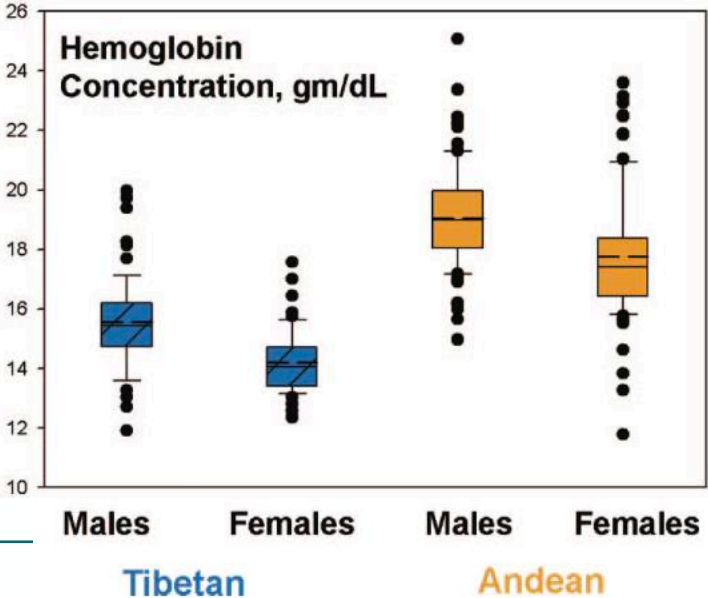
	Sojourners	Tibetans	Andeans
Resting ventilation	↑	↑	—
Hb concentration	↑		
Blood flow	↑		
Capillary density	↑		
Birth weight Offspring survival	↓		



Adaptive responses to high-altitude hypoxia

Homeostasis + acclimation

	Sojourners	Tibetans	Andeans
Resting ventilation	↑	↑	—
Hb concentration	↑	—	↑
Blood flow	↑		
Capillary density	↑		
Birth weight Offspring survival	↓		

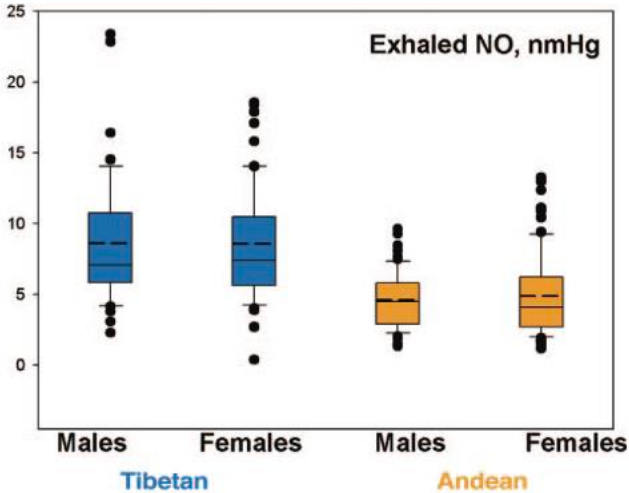


Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

	Sojourners	Tibetans	Andeans
Resting ventilation	↑	↑	—
Hb concentration	↑	—	↑
Blood flow	↑	↑ ↑	↓
Capillary density	↑		
Birth weight Offspring survival	↓		

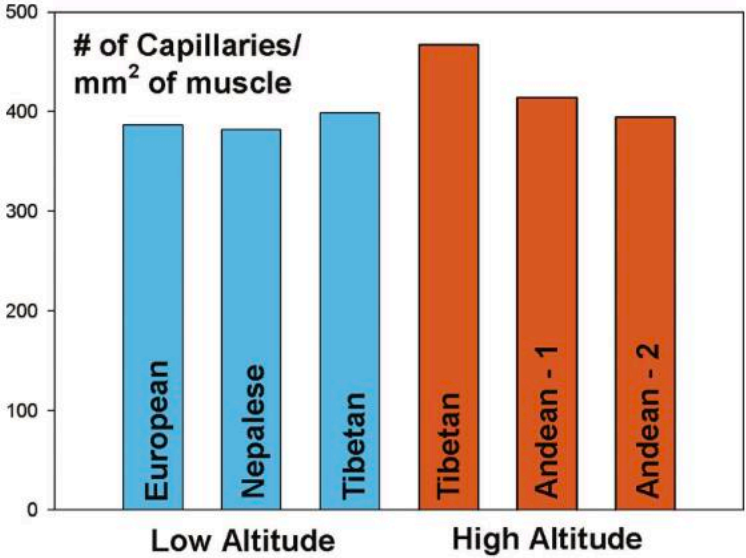
Tibetan placentas:
larger arteries,
permitting higher
blood flow



Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

	Sojourners	Tibetans	Andeans
Resting ventilation	↑	↑	—
Hb concentration	↑	—	↑
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Birth weight Offspring survival	↓	—	—



Adaptive responses to high-altitude hypoxia

Homeostasis +
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Resting ventilation	↑	↑	—
Hb concentration	↑	—	↑
Blood flow	↑	↑ ↑	↓
Capillary density	↑	↑ ↑	↑
Stature			
Lung volume			
Birth weight Offspring survival	↓	—	—

Adaptive responses to high-altitude hypoxia

Homeostasis +
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Capillary density	↑	↑ ↑	↑
Stature	—	↓	↓
Lung volume	—	↑	↑
Birth weight Offspring survival	↓	—	—

Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

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Resting ventilation	↑	↑	—
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Capillary density	↑	↑ ↑	↑
Stature		↓	↓
Lung volume		↑	↑
Birth weight Offspring survival	↓	—	—

Tibetans:

Dampened erythrocytosis
acclimation
response

Strategy based
on blood flow

Andeans:

Strategy based
on
erythrocytosis

Both: changed
growth patterns

Not enough
time for other
responses

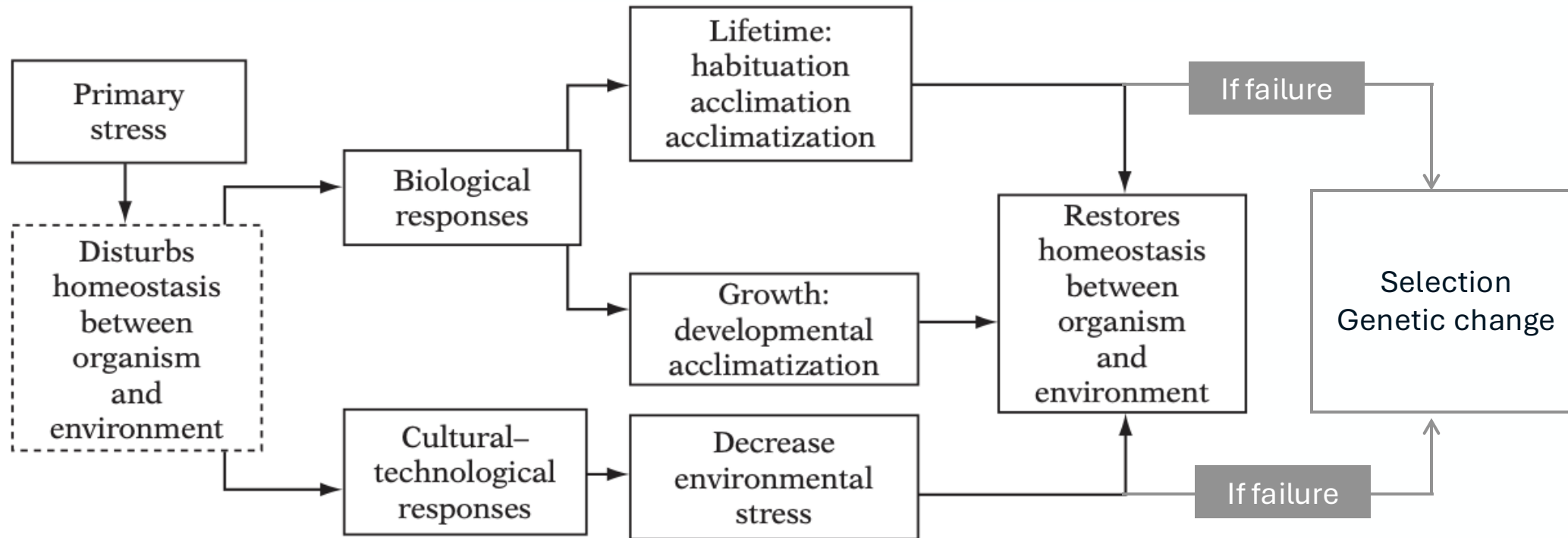
VS

Recall lecture 3

The problem

Your solutions

Natural selection's solutions

**Both: changed growth patterns**

Hard to tease apart genetic and developmental adaptation

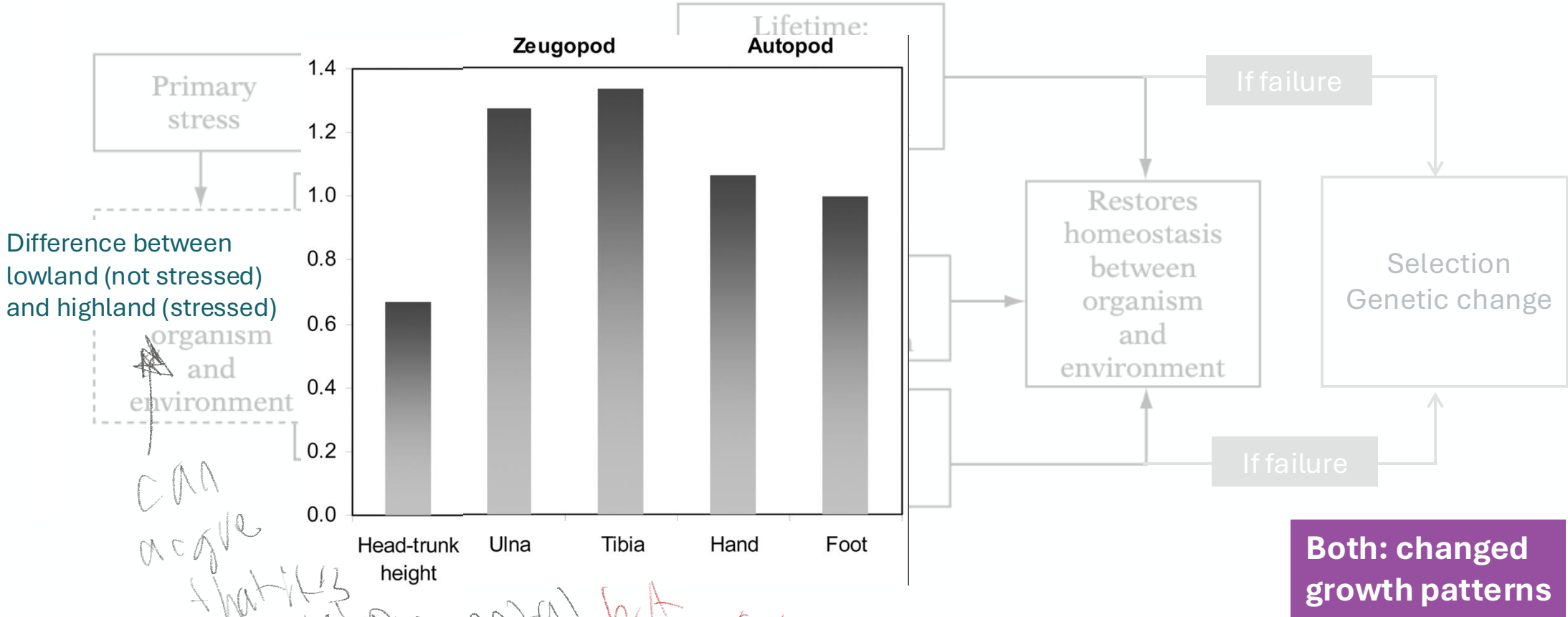
Open question: is this genetic (reflecting long-term adaptation through natural selection) or developmental plasticity?

Recall lecture 3

The problem

Your solutions

Natural selection's solutions



can argue that it's developmental but...

Open question: is this genetic (reflecting long-term adaptation through natural selection) or **developmental plasticity**?

Recall lecture 3

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Natural selection's solutions

There is genetic evidence for selection on *Andeans* on growth

Primary stress

Lifetime: habituation acclimation acclimatization

If failure

Selection Genetic change

If failure

Both: changed growth patterns

Article

A positively selected *FBN1* missense variant reduces height in Peruvian individuals

<https://doi.org/10.1038/s41586-020-2302-0>

Received: 28 February 2019

Accepted: 10 March 2020

Published online: 13 May 2020

 Check for updates

Samira Asgari^{1,2,3,4,5}, Yang Luo^{1,2,3,4,5}, Ali Akbari^{4,6}, Gillian M. Belbin^{7,8,9}, Xinyi Li^{1,2,3,4,5}, Daniel N. Harris^{10,11}, Martin Selig¹², Eric Bartell^{4,5,13}, Roger Calderon¹⁴, Kamil Slowikowski^{1,2,3,4,5}, Carmen Contreras¹⁴, Rosa Yataco¹⁴, Jerome T. Galea¹⁵, Judith Jimenez¹⁴, Julia M. Coit¹⁶, Chandel Farroñay¹⁴, Rosalynn M. Nazarian¹², Timothy D. O'Connor^{10,17}, Harry C. Dietz^{18,19}, Joel N. Hirschhorn^{4,6,13,20}, Heinner Guio²¹, Leonid Lecca¹⁴, Eimear E. Kenny^{7,8,9}, Esther E. Freeman²², Megan B. Murray¹⁶ & Soumya Raychaudhuri^{1,2,3,4,5,23} ✉

Open question: is this **genetic** (reflecting long-term adaptation through natural selection) or developmental plasticity?

Adaptive responses to high-altitude hypoxia

Homeostasis +
acclimation

	Sojourners	Tibetans	Andeans
Resting ventilation	↑	↑	—
Hb concentration	↑	—	↑
Blood flow	↑	↑ ↑	↓
Capillary density	↑	↑ ↑	↑
Stature		↓	↓
Lung volume		↑	↑
Birth weight Offspring survival	↓	—	—

Tibetans:

Dampened
erythrocytosis
acclimation
response

Strategy based
on blood flow

Andeans:

Strategy based
on
erythrocytosis

Both: changed
growth patterns

Which of these responses is underpinned by genetics?

Tibetans:

Dampened
erythrocytosis
acclimation
response

Strategy based
on blood flow

Andeans:

Strategy based
on
erythrocytosis

Which of these responses is underpinned by genetics?

Tibetans:

Dampened
erythrocytosis
acclimation
response

Strategy based
on blood flow

Andeans:

Strategy based
on
erythrocytosis

Hypoxia-inducible factors

EGLN1

PHD2

AKO selection on/around here

Natural selection on *EPAS1* (*HIF2α*) associated with low hemoglobin concentration in Tibetan highlanders

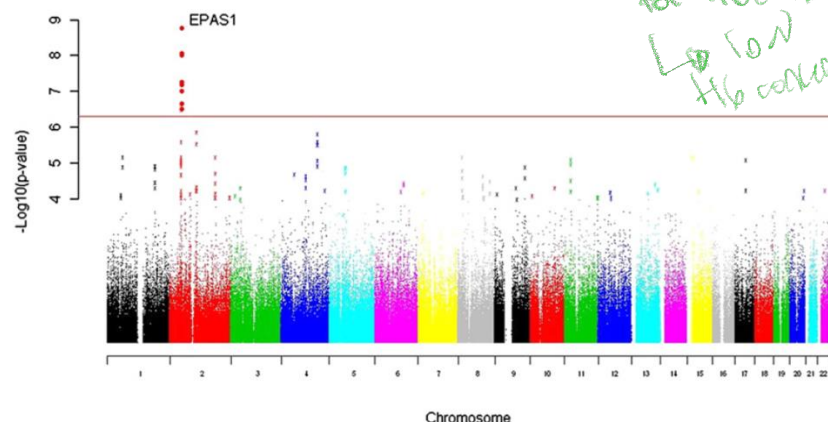
Cynthia M. Beall^{a,1}, Gianpiero L. Cavalleri^{b,1}, Libin Deng^{c,2}, Robert C. Elston^d, Yang Gao^c, Jo Knight^{e,f}, Chaohua Li^c, Jiang Chuan Li^g, Yu Liang^h, Mark McCormack^b, Hugh E. Montgomery^{i,1}, Hao Pan^c, Peter A. Robbins^{j,1,3}, Kevin V. Shianna^k, Siu Cheung Tam^l, Ngodrop Tsering^m, Krishna R. Veeramahⁿ, Wei Wang^h, Puchung Wangdui^m, Michael E. Weale^{e,1}, Yaomin Xu^o, Zhe Xu^c, Ling Yang^h, M. Justin Zaman^p, Changqing Zeng^{c,1,3}, Li Zhang^{o,1}, Xianglong Zhang^c, Pingcui Zhaxi^{h,1,4}, and Yong Tang Zheng^q

EPAS1

HIF2A

HIF3A

Signals of natural selection for Tibetans Low Hb concentration



Transcription factors

Tibetans:

Dampened erythrocytosis **acclimation** response

Erythropoietin
(erythrocytosis)

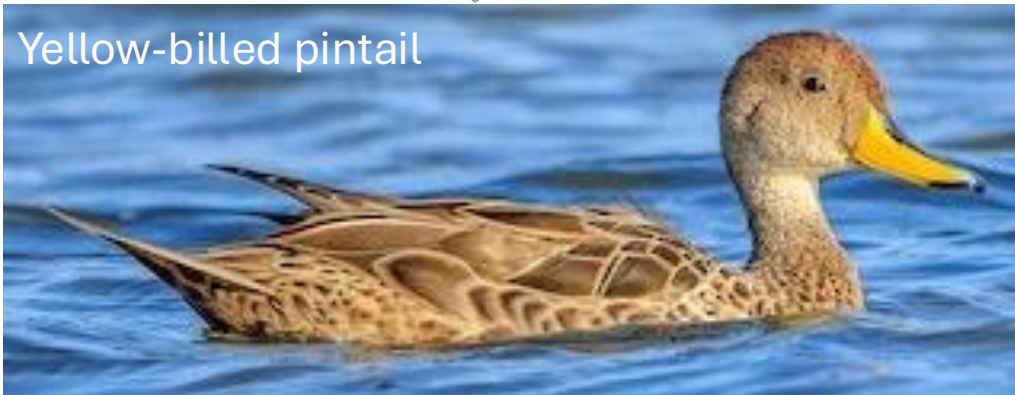
Transferin
(transfer of iron, necessary for haemoglobin)

Target genes/proteins

Hypoxia-inducible factors

birds also have these adaptations

Yellow-billed pintail



Cinnamon teal



Speckled teal



EGLN1

PHD2

dependent on

HIF1A

HIF2A

HIF3A

Transcription factors

Tibetans:

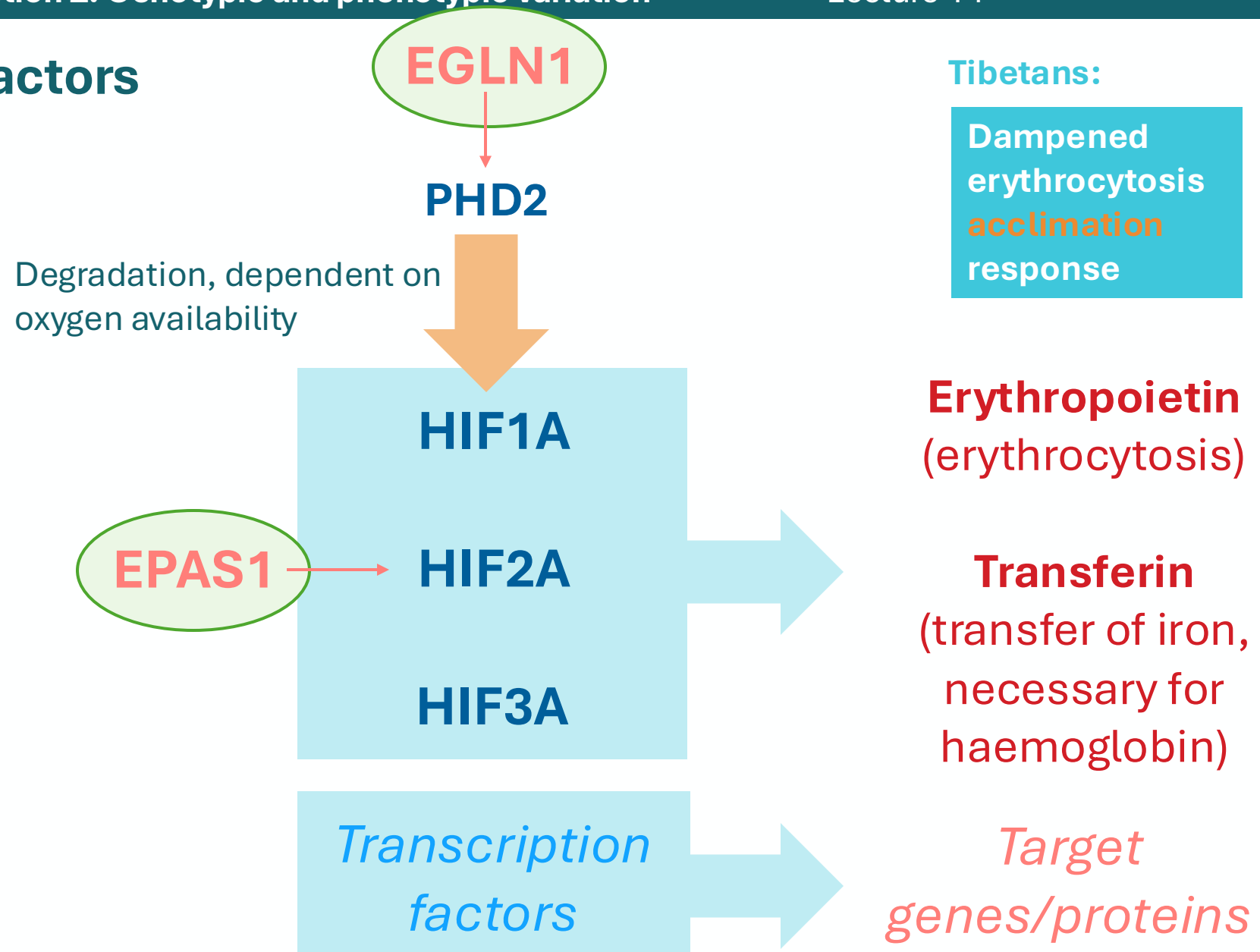
Dampened
erythrocytosis
acclimation
response

Erythropoietin
(erythrocytosis)

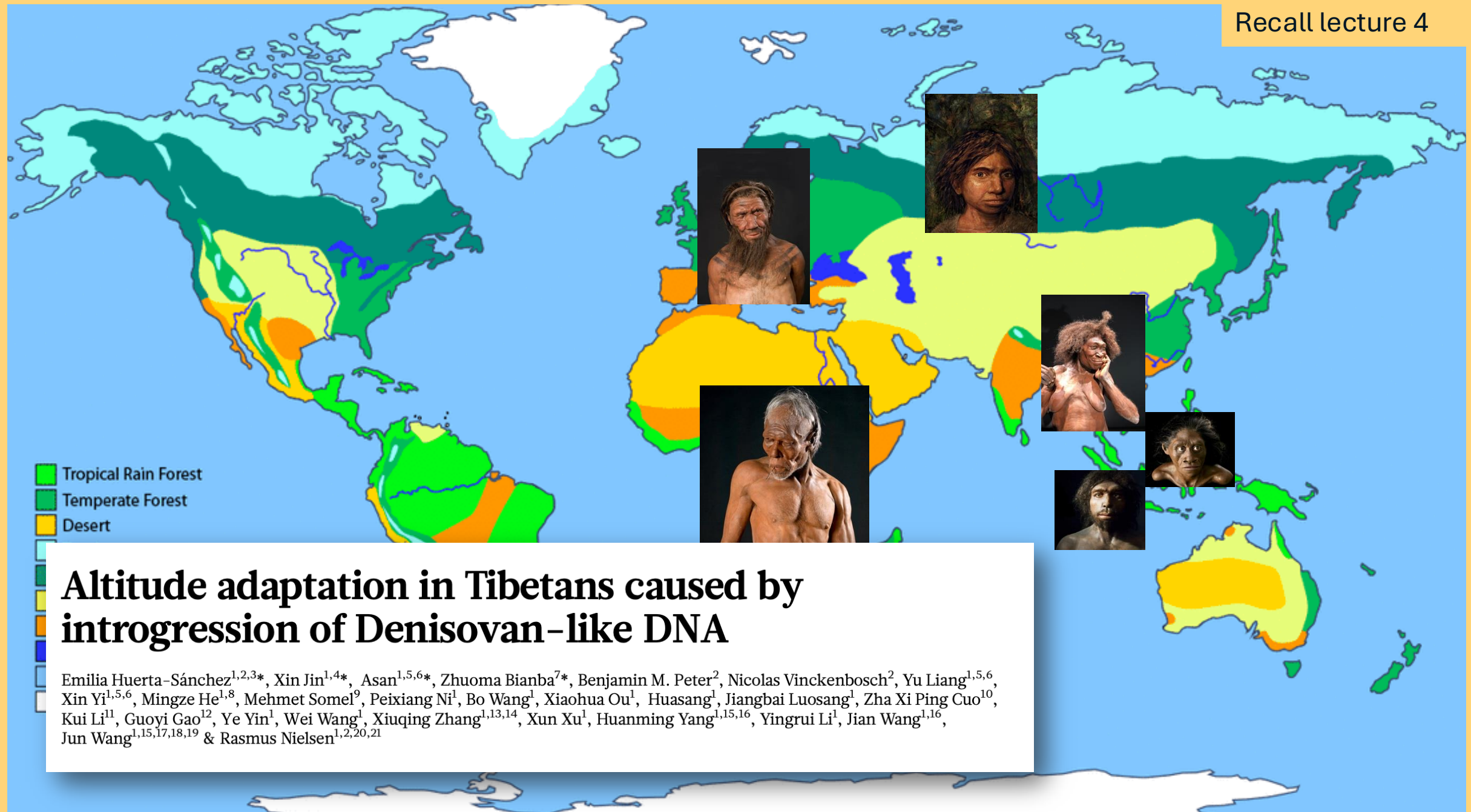
Transferin
(transfer of iron,
necessary for
haemoglobin)

*Target
genes/proteins*

Hypoxia-inducible factors



Recall lecture 4



Drawing on knowledge from past lectures...

28 Aug	Introduction and key frameworks
2 Sep	Adaptation, evolution, and diversity I
4 Sep	Adaptation, evolution, and diversity II
5 Sep	
9 Sep	The deep roots of human biology I
11 Sep	The deep roots of human biology II
12 Sep	
16 Sep	Studying our species: history and ethics
18 Sep	Genetic diversity in modern humans
19 Sep	
23 Sep	Local adaptation
25 Sep	Culture and genetic diversity
26 Sep	
30 Sep	Genes≠phenotypes
2 Oct	Body shape and size: adaptations to temperature stress?
3 Oct	
7 Oct	Skin and hair: adaptations to solar radiation?

Adaptation framework:

- Hypoxia an extreme stressor
- Homeostatic & acclimating responses
- Growth: Developmental plasticity vs genetics

Deeply conserved pathways
Out-of-Africa & meeting other species

Great example of local adaptation
Alongside disease, hypoxia the strongest selective factor

Growth: developmental plasticity vs genetics

Drawing on knowledge from past lectures...and building on them

Key

28 Aug	Introduction and key frameworks
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Adaptation framework:

- Hypoxia an extreme stressor
 - Homeostatic & acclimating responses
 - Growth: Developmental plasticity vs genetics
- Maladaptive

Deeply conserved pathways
Out-of-Africa & meeting other species

↳ Denisovan admix in Tibetans

Great example of local adaptation
Alongside disease, hypoxia the strongest selective factor

→ Divergence

↑
Due to amount of time to evolve adaptations

Growth: developmental plasticity vs genetics

The problem

Your solutions

Natural selection's solutions